

The Uninsured Decade

The one catastrophic risk you cannot buy a policy against — and the premium you can still pay

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Who This Book Is For

This is a book about money before it is a book about health. That is deliberate.

It is for the person in the household who reads the statements. Who carries the umbrella policy, the key-man coverage, the term life he has never needed and pays for anyway, because that is what the responsible one does — buys down risk for people who are not thinking about risk. In my experience that is usually a man in his fifties or sixties, and I write from that seat. It is not always. If it is you, this book is for you.

You will not find inspiration in here. No transformation stories, no before-and-after photographs, no promises that a supplement will save you. What you will find is an exposure your plan does not cover, priced out with sources, and one honest way to reduce the odds — with the odds stated as odds, never as a guarantee.

A word to the spouse. You may be the one who bought this. If so, you already know what happens in the ten years after a diagnosis, because you have watched it happen in another family and you have been unable to get the conversation started in your own. Do not open with your worry. Hand it over and

say one sentence: *Read the first six pages. If the numbers are wrong, throw it away.*

The numbers are not wrong. Every figure in this book carries its source and its date. Check them.

— **Dr. TJ**

A Note on the Numbers

I am going to ask you to make a decision about money, so I owe you a clean audit trail.

Every figure in this book carries a source and a year, in the text, where you read it. Not in an endnote you will never flip to. If a number is from 2024 and you are reading this in 2027, you should be able to see that on the page and discount it accordingly. The full citation list is in the back, and the working version — updated when the underlying surveys are — lives at my4mlife.com/science.

Some of these figures are older than they look. Care-cost surveys run on an annual cycle; national dementia-cost models run on a longer one. Where a widely quoted number rests on data that is several years stale, I say so in the sentence rather than letting the freshness of the citation imply the freshness of the data. You will see me do this with the caregiver out-of-pocket figure, which is 2021 data still in circulation, and with a 1999 mortality study that remains the best evidence we have on what caregiving does to the caregiver.

Where the sources disagree, I do not average them.

There are two respectable national models of what dementia costs America, built on different definitions, and they produce very different totals. They are not additive and I do not add them. Where I have to choose, I choose the more conservative one and tell you that I did.

Bill and Cindy are a composite. The couple you are about to meet is not one household. They are assembled out of the arc that shows up over and over in the national data and in thirty years of watching families do this — the diagnosis in the early seventies, the eight-to-ten-year decline, the money leaving in three distinct stages. Every dollar figure attached to them comes from a published survey of what that care actually costs. The tennis league is invented. The invoice is not.

One number I will not give you is your number. The ten-year ledger in Chapter 4 is a national-median household. Yours will be higher or lower depending on the state you live in, the house you own, the years you get, and whether there is a daughter within driving distance. If you want your own figure, run it — my4mlife.com has a calculator that takes your inputs and does this arithmetic on your household instead of a composite one.

That is the standard. Hold me to it.

Part I — The Bill

Chapter 1 — The Risk You Didn't Underwrite

Somewhere in your house there is a folder, or a portal login, that holds every policy you own.

Homeowner's, with the wind-and-hail rider you added after the storm two summers ago. Auto, on three vehicles, one of them your daughter's. An umbrella policy over the top of both, because your attorney told you to and he was right. Term life — twenty-year, maybe a second one layered on it, bought when the kids were small and now mostly a habit. Disability, if you were the sort of earner whose absence would matter, which you were. Key-man on the business. Errors and omissions. Workers' comp. Maybe a buy-sell agreement funded by a policy nobody has looked at in nine years.

You did that. Not your wife, not your accountant — you. Over thirty years you built a lattice of coverage under your family so that nothing that could happen to you would land on them as a bill. That is not paranoia. That is the whole job. Most of the men I have known who did it well would not describe them-

selves as careful people. They would describe themselves as people who did not want anyone else cleaning up after them.

So let me ask you a question about that folder.

Which policy in it pays for memory care?

Take a second with it. Not the hospital stay — you have that. Not the rehab after the hip — Medicare covers a stretch of that, and we will get to exactly how long. I mean the other thing. The thing that starts as a word you cannot find in a meeting and ends, eight or ten years later, with a bed in a locked wing and someone else spooning food into your mouth at a rate of eight thousand dollars a month, every month, until it stops.

Which policy covers that one?

I already know the answer, because I have asked this question in a lot of rooms now. There is a pause. Then somebody says *Medicare*, half as a statement and half as a question, and everyone else at the table looks relieved for about four seconds, until they see that I am not nodding.

Medicare pays nothing toward it. Not a reduced amount. Not a copay structure with a bad deductible. Zero. It is the single largest uncovered financial exposure in American retirement, it sits in almost every household in this country, and the reason nobody has sold you a solution is that there is barely a solution left to sell.

I want to be careful here, because you have been sold things before and you have a good instinct for when it is happening.

I am not going to tell you that you can prevent dementia. Nobody can tell you that. Anyone who does is either careless or selling something, and by the end of Chapter 9 you will be able to hear the difference in one sentence. What I am going to tell you is that this is a risk, that risk is a thing you already know how to price, and that there is exactly one place left to post a reserve against it.

Not a carrier. Not a rider. You.

Let's start with what it costs when nobody does.

First, the inventory

Before we price the exposure, price the coverage. Do this on paper. It takes four minutes and it is the only exercise in this chapter.

Risk	Policy you hold	Who writes the check
House burns	Homeowner's + rider	Carrier
Someone is hurt on your property	Homeowner's + umbrella	Carrier
Car wreck, at fault	Auto + umbrella	Carrier
You die early	Term life	Carrier
You can't work at 55	Long-term disability	Carrier
Your partner dies, business at risk	Key-man / buy-sell	Carrier
Professional mistake	E&O	Carrier
Hospitalization at 72	Medicare + supplement	Carrier
Rehab after the hip, first 100 days	Medicare Part A, conditions apply	Carrier, then you
Ongoing custodial and memory care	none available	You. Then your spouse. Then Medicaid.

Every line but one resolves to somebody else's balance sheet. That was the point of thirty years of premiums — to move the tail risk off your family and onto an institution built to absorb it.

The last line does not resolve. It is not underinsured. It is uninsured, and for most people reading this it is uninsurable, and

Chapter 3 is the unhappy story of why.

Notice something about the shape of that table. The risks you covered are, almost without exception, the low-probability ones. Your house probably will not burn. You probably will not be sued past your auto limits. You bought those policies anyway, because the loss, if it landed, would be unrecoverable — and unrecoverable is the test, not likely.

Now look at the uncovered line and ask what its probability actually is. A 2025 study in *Nature Medicine* — the largest lifetime-risk analysis of its kind, following more than 15,000 people from age 55 until they either developed dementia or died — puts the lifetime risk of dementia after 55 at **42%**: 35% for men, 48% for women [Fang et al., Nat. Med. 2025]. That is not a scare number. It is a coin flip that leans against you. In a marriage, you are not underwriting one life. You are underwriting the question *does this happen to either of us* — and run that arithmetic and the odds come out to roughly two in three. Not “meaningfully worse.” Worse by a margin that should change how you read every other line in this chapter.

You insured the fire. You did not insure the likelier thing.

And you did not do it out of carelessness. You did it because in every other row of that table, somebody eventually came to your office, or called you, or sat across a conference table with an illustration and a pen and asked you to sign. Coverage does not get purchased because a buyer went looking. It gets purchased because a market showed up. On the last row, no mar-

ket showed up — and Chapter 3 is the unhappy story of where it went.

One more thing about the table before we leave it. Look at the third column on the last row and notice that it has three entries, in sequence, and that they are not alternatives. They are the order of operations. First you pay. Then, when you are no longer able to make decisions, your spouse pays out of the same accounts. Then, when the accounts will not carry it, a means-tested government program takes over — and the qualifying event for that program is that the money is gone.

That is the actual coverage structure on the largest exposure in your retirement. Self-pay, then spouse-pay, then impoverishment. It has been sitting in that folder the whole time with nothing written next to it.

Bill

Bill is seventy-one. He is a composite, and I will keep saying so, but he is assembled out of a hundred versions of the same man.

He sold his share of a mechanical contracting business at sixty-eight, which was two years later than he had told everyone he would. He plays doubles on Tuesdays and Thursdays in a league where he is neither the best nor the oldest player, and he is quietly proud of both facts. He drives a nine-year-old truck he keeps meaning to trade. He and Cindy have been mar-

ried forty-four years. Two kids, one in Dallas, one four hours away. Four grandchildren, whose school pictures are held to the refrigerator with magnets from a bank that no longer exists.

Two years ago Bill tore his rotator cuff reaching for a low forehand. He had it repaired six weeks later at a good orthopedic practice, spent five months in physical therapy, and was back on the court by the following spring with a serve he complains about. Total out-of-pocket, all in: a few thousand dollars. Medicare and the supplement absorbed the rest, exactly as designed. Bill barely thinks about it now except when the weather changes.

That is what a covered risk looks like from the inside. It is expensive, it is disruptive, it hurts, and then it ends, and the household balance sheet closes back up over the wound.

So let me ask the harder version of the question.

What if it isn't Bill's shoulder? What if it's Bill's memory?

A rotator cuff changes your tennis game. Cognitive decline changes who pays the bills, who drives, who remembers which grandchild is which — and it hands Cindy a job she never applied for: nurse, accountant, guardian, and eventually the only one in the marriage who remembers the marriage.

The shoulder was insured. The memory is not.

This is an audit finding, not a scare

I am aware of what I am doing to you on page five, and I want to name it.

There is an entire industry that makes its living getting men your age frightened about their brains, and then selling them a bottle. I am not in that business and this is not that book. What I have handed you so far is not a prognosis. It is an audit finding — one line item, on a coverage schedule you built yourself, that has nothing in the second column.

You already know how to respond to an audit finding. You do not panic at it and you do not ignore it. You size it, you decide whether it is material, and if it is, you fund it.

So that is the order of this book. Part I sizes the exposure — what Medicare actually pays, why the private market that used to cover this walked away, and what ten years costs a household, year by year, in dollars you can check. Part II prices the only premium still available to you, in hours and in dollars, with nothing hidden. Part III is what that premium buys, stated honestly, including the part where I tell you plainly what it does not buy. Part IV is thirty days.

One promise before we go on. I am not going to tell you at the end that this pays for itself. When I run the expected-value math in front of you later — and I will run it in front of you, with the arithmetic showing — it does not come out to a clean

break-even against dementia costs alone. With the odds at roughly two in three for a couple, that math gets close — close enough that a conservative, evidence-supported risk reduction gets a household most of the way to the number, which is closer than it has any right to be. But close is not break-even, and I am not going to round it up for you. Anyone who tells you otherwise is doing sales, not math. There is a real case for paying this premium and I think it is a strong one, but it is not the case that you will make your money back on dementia-cost avoidance alone, and I would rather lose you in Chapter 9 than get you there on a number I had to bend.

Start with the coverage read.

Chapter 2 — What Medicare Actually Covers

The most expensive assumption in American retirement planning is that Medicare handles this.

It does not. Not partially. Not with a bad deductible. This chapter is going to be unemotional, because it is not an argument — it is a coverage read, the same read your risk manager would do on any policy before you signed it. I am going to show you exactly where coverage stops and your checkbook starts.

The sentence itself

Here is the operative language, from Medicare's own page on long-term care:

“Medicare doesn’t cover custodial care if it’s the only care you need.”

[Medicare.gov, current]

The statutory machinery underneath it is 42 CFR §411.400, which excludes payment for custodial care. That regulation is not new, it is not under active reform, and it is not a loophole somebody is about to close in your favor.

Everything turns on one word, so let's define it the way the program does.

Skilled care is care that requires a licensed professional to deliver — wound management, IV therapy, physical therapy after a joint replacement, rehabilitation with a documented trajectory of improvement. Medicare covers skilled care.

Custodial care is help with the activities of daily living: bathing, dressing, toileting, transferring from a bed to a chair, eating, and supervision to keep a person from wandering out the front door at two in the morning. It does not require a nurse. It requires a person, in the room, for as many hours a day as the disease demands.

Read that second definition again and notice something. It is a nearly exact description of what dementia care *is*, from the middle stage onward. The entire disease course, from the day the spouse can no longer manage alone until the last day, is delivered almost entirely as custodial care.

Which means the category of care this disease actually needs is the specific category the program specifically excludes. That is not an oversight. Medicare was designed in 1965 as acute med-

ical insurance. It does what it was built to do. It was never built to do this.

What is covered — and the 100 days everyone misremembers

To be fair to the program, here is the honest two-column read.

Medicare does cover	Medicare does not cover
Hospitalization	Ongoing memory care
Physician visits, including the neurologist	Assisted living room and board
Diagnostic imaging and cognitive testing	In-home aides for supervision or bathing
Prescription drugs, under Part D	Adult day programs, generally
Hospice, at the very end	Long-term nursing home custodial stays
Up to 100 days of skilled nursing per benefit period, conditions apply	Anything that is “only” custodial

That last covered line is the one that does the most damage, because people remember the number and forget the conditions.

The 100 days is not 100 days of nursing home. It is up to 100 days of *skilled* nursing facility care per benefit period, and it

comes with gates. You generally need a qualifying inpatient hospital stay first. The facility care has to be skilled, and it has to remain skilled — meaning a clinician has to keep documenting that skilled services are medically necessary. And even inside those 100 days, the coverage steps down: the first 20 days are paid in full, and from day 21 through day 100 there is a substantial daily coinsurance the patient owes unless a supplement picks it up.

In practice, for a dementia course, the 100 days is usually irrelevant. It gets consumed by something else — the hip fracture, the pneumonia, the post-hospital rehab — and then the person goes home, or goes to memory care, and the coverage conversation is over. The disease itself does not qualify. There is no benefit period that restarts for “still has dementia.”

So put the number in its place. It is not a hundred days of long-term care. It is a rehab benefit that happens to be measured in days, and it does not attach to this.

What the private-pay meter reads

If Medicare is not paying, someone is. Here is what that someone pays, at national medians, with the year on each figure.

- **Memory care:** \$8,019 per month, national median, 2026 — about \$96,000 a year. In Texas, \$6,684 [SeniorLiving.org 2026].

- **Assisted living:** \$6,200 per month, or \$74,400 a year, up 5% year over year [CareScout 2025].
- **In-home caregiver:** \$35 an hour, national median. At 20 hours a week that is \$36,400 a year; at something approaching full-time coverage, 44 hours a week, it is \$80,080 [CareScout 2025].
- **Nursing home, private room:** \$129,575 a year — \$355 a day [CareScout 2025].

None of those are the sticker price at the nice place near your daughter. They are national medians. Half the country pays more.

Two things about those figures that people consistently get wrong.

The first is that they are not stable. Assisted living rose 5% year over year in the most recent survey [CareScout 2025]. Care is labor, labor is the scarcest input in this economy, and there is no version of the next fifteen years in which the wage floor under a memory-care aide goes down. Whatever number you are looking at is the beginning of the series, not the middle of it.

The second is that the household almost never buys only one of these. The arc runs through several. The in-home aide comes first and gets extended, hour by hour, until the arithmetic of the aide and the arithmetic of the facility cross. Then there is a facility, and the facility bill is not the whole facility bill — memory-care communities commonly price a base rate

plus a care level, and the care level goes up as the disease does. The quoted median is the number on the first invoice.

And notice what the numbers do as the disease advances. Early on, the household is paying for medications, visits, home modifications, and legal work — annoying, absorbable. In the middle, it is paying for hours of another human being’s time. At the end, it is paying for a bed with staff attached to it around the clock. The bill does not spike. It escalates on a schedule, which is what makes it so easy not to see coming.

Spend-down: the coverage that exists, and what it costs to qualify for it

There is one government program that pays for long-term custodial care, and it is not Medicare. It is Medicaid — and Medicaid is means-tested, which means the way you become eligible is by not having money anymore.

The industry word for that process is **spend-down**. It is worth knowing the term, because it is what your estate attorney will call it and because it is one of the few places in American financial life where the plan is, explicitly, to run out.

The specific numbers vary by state, and 2026 Texas figures are illustrative rather than universal, but the architecture is the same nearly everywhere [TX Medicaid, 2026 figures]:

- **Countable resources** must come down to roughly \$2,000 for an individual applicant, or \$3,000 for a couple.
- There is a **five-year, sixty-month look-back** on asset transfers. Gifts inside that window — the down payment you helped your son with, the money you moved to a trust when you finally got around to it — trigger a penalty period of ineligibility. This is the provision that punishes families who start planning after the diagnosis instead of before it.
- The at-home spouse is not left with nothing. A **Community Spouse Resource Allowance** protects somewhere between \$32,532 and \$162,660 in assets, plus up to \$4,066.50 a month in protected income.

Look at that last bullet honestly. It is a real protection, and it is the reason Cindy does not end up on a friend's couch. It is also, at the top of the range, roughly the assets a careful household accumulates in a decade of a good career — and it is what remains, at seventy-nine, of a retirement they spent thirty years building.

That is the coverage read. There is one program that pays, its eligibility requirement is impoverishment, and the waiting room is your own balance sheet.

You would buy the private policy instead, if you could.

For a while, you could.

Chapter 3 — The Market That Left

You are not the first person to notice the gap.

An entire industry noticed it in the 1990s, priced it, sold it hard, and then discovered it had gotten the pricing badly wrong. What happened next is why your insurance agent — a competent person who has sold you six other products — has probably never brought this one up.

This chapter is going to be honest about a market I do not participate in and cannot advise you on. Nothing here is a recommendation to buy, drop, or change any policy. I am telling you what happened to the market because the state of that market is the reason the rest of this book exists.

Everybody left

Around the year 2000, more than a hundred carriers sold standalone long-term-care insurance in the United States. Today fewer than a dozen actively write new standalone policies [Am. Academy of Actuaries].

That is not a market correction. That is an evacuation.

The reason is straightforward, and if you have ever priced risk for a living you will recognize it as the classic way to get killed. The original policies were written on three assumptions, all of which turned out to be wrong in the same direction. Carriers assumed a meaningful share of policyholders would lapse — stop paying, walk away, free up reserves. They assumed interest rates on the invested premium float would stay in a range that made the math work. And they assumed a certain claims frequency and duration.

Almost nobody lapsed, because people who buy this product buy it deliberately and hang on. Rates fell for two decades and stayed down. And people lived longer with conditions that required years of care rather than months.

Three assumptions, all wrong, all compounding on the same side of the ledger. The block went underwater and stayed there.

What happened to the people who already bought it

Here is the part that stings, and the part your neighbor with the policy will tell you about if you ask him.

Most of these contracts are guaranteed renewable but not premium-guaranteed. The carrier cannot cancel you. It can go to

your state insurance department and ask to raise your rate on the entire class of policies — and it has, repeatedly.

The public filings are not subtle. Genworth sought and received a 68.5% average approved increase in a 2026 Nebraska filing; in 2022 it raised rates on more than two thousand policyholders by an average of 97%, with individual increases ranging from 79% to 173%; a 161% request was denied in Massachusetts as “unjust, unfair and inequitable” [S&P Global; state filings, 2022–2026]. John Hancock secured increases across more than ten states in a single 2026 quarter, adding roughly \$35.1 million in annual premium — on top of a cumulative history on some blocks that includes 15% in 2012–15, 32.3% in 2017, 43.8% phased over three years starting in 2020, and another 15% in 2024 [S&P Global; longtermcare-desk.com, 2026].

Stack those. A seventy-year-old on one of those blocks may be paying two or three times what he signed up for at fifty-eight, at exactly the point in life when his income is fixed. He has three options and he knows all of them: pay it, reduce his benefit, or lapse a policy he has funded for twelve years and walk away with nothing.

That is not a rogue carrier. That is a whole product category repricing a mistake onto the people who bought it in good faith.

The window that closes before you shop

Now the underwriting problem, which is the one that actually applies to you.

Long-term-care coverage is medically underwritten, and the underwriting is not a formality. Declines are common, and the conditions that get you declined — cognitive testing that comes back soft, an existing neurological diagnosis, poorly controlled diabetes, a recent cardiac event, mobility limitations — are precisely the conditions that make a man start thinking he ought to look into long-term-care coverage.

That is the trap in one line. **The event that makes you want the policy is the event that makes you uninsurable for it.**

And the pricing window is narrow on the other end too. A healthy sixty-year-old couple buying moderate coverage today — roughly \$165,000 of initial benefit with 3% growth — is looking at something in the range of \$4,600 to \$7,200 a year combined, with reduced-coverage bundled quotes as low as \$2,550 to \$4,675 [industry pricing data, July 2026]. Wait until sixty-eight and the premium is materially higher and the health questions are materially harder. Wait until the first missed word and it may not be available at any price.

So the honest count. Only about 10 to 15% of Americans 65 and older hold long-term-care insurance of any kind — and

only 3 to 4% of those over 50 are currently paying premiums on it — against an estimate that roughly 70% of people over 65 will need long-term care of some sort [CRR / HCG Secure, 2026].

Read those two numbers next to each other. Seventy percent will need it. Ten to fifteen percent have it.

You are not lazy for being in the uncovered majority. You are in a market that priced most people out and underwrote the rest of them away.

What is left on the shelf

The product that has partly filled the vacuum is the hybrid — life insurance or an annuity with a long-term-care rider attached, so the money goes somewhere either way and the “use it or lose it” objection disappears. Industry research describes fixed hybrid life-with-LTC products as the leading product trend of 2026, growing as standalone carriers exit, with a majority of surveyed consumers saying they want coverage of this shape [LIMRA 2026].

These are real products and some people should own them. They also, as a rule, cap out well below the ten-year exposure in the next chapter — the LTC benefit is typically a multiple of a death benefit you were partly buying anyway, not an open-ended pool sized to a decade of memory care.

If you are going to shop it anyway — three questions. I do not sell insurance, I cannot advise you on it, and you should take this to a licensed agent and to your own advisors. But if you do go looking, these are the three questions I would want answered in writing:

1. **Is the premium guaranteed, or guaranteed renewable?** Those are not the same sentence. One means the price is fixed. The other means they cannot cancel you but they can reprice the class. Ask which one you are signing, and ask for the carrier's actual rate-increase history on its existing blocks — not its intentions.

2. **What exactly triggers a benefit, and who decides?** Most contracts pay on inability to perform two of six activities of daily living, or on a documented cognitive impairment. Ask how cognitive impairment is assessed, by whom, and what the elimination period is — the deductible measured in days, during which you are paying the full private rate yourself.

3. **What does the benefit look like in the twelfth year, in real dollars?** A daily benefit with no inflation rider is a shrinking asset against a bill that is escalating. Ask for the benefit stated against the projected cost of care in the year you would most likely use it, not against today's.

Ask those three and you will know more than most buyers do at signing.

And then come back here, because whatever you buy will cover part of it, and this chapter's real conclusion is the one nobody at the kitchen table wants to say out loud: for most households reading this, there is no carrier standing behind the largest financial exposure in their retirement.

So let's find out what that exposure actually is.

Chapter 4 — Ten Years, One Household

Bill is diagnosed at seventy.

It does not happen in a dramatic way. It happens the way it usually happens: eighteen months of Cindy noticing, six months of Cindy not saying anything, one bad afternoon when he cannot find his way back from the club he has driven to twice a week for eleven years, and then a neurologist's office in a building with a parking garage, and a word.

People with Alzheimer's commonly live eight to ten years after diagnosis. So the question in front of this household is not *what does a diagnosis cost*. A diagnosis costs a copay. The question is what the decade costs.

Here is the decade, in installments.

The ten-year ledger

Stage	What the care actually looks like	Estimated family cost
Years 1–3 (age 70–73) Early	Cindy provides nearly all care. Doctor visits, medications, home safety changes, legal and financial planning. Average dementia-caregiver out-of-pocket spending runs about \$12,388 a year — <i>a figure based on 2021 data, and meaningfully higher in today’s dollars</i> [Alz. Impact Movement, 2021 data].	~\$35,000–\$40,000
Years 4–6 (age 73–76) Middle	Cindy can no longer do it alone. Paid in-home caregiver 20 hours a week at \$35/hr (~\$36,400/yr) [CareScout 2025], plus an adult day program two days a week (~\$9,900/yr), plus out-of-pocket medical.	~\$170,000–\$180,000
Years 7–10 (age 76–80) Late	24/7 supervision required. Memory care runs \$8,019/mo nationally, about \$96,000 a year [SeniorLiving.org 2026]; a private nursing-home room is \$129,575 a year [CareScout 2025]. Assume roughly 3.5 years in a facility.	~\$290,000–\$340,000
Ten-year total	Cash and care costs falling on the household — before counting the healthy spouse’s unpaid hours, lost income, or health decline.	~\$495,000–\$560,000

Sit with the bottom line for a second and then let me tell you why I believe it, because a half-million-dollar number invites

the reasonable suspicion that somebody built it backward from the number they wanted.

The Alzheimer's Association puts the average total lifetime cost of care for one person with dementia at **\$405,262**, in 2024 dollars, with **70% of it borne by the family** through out-of-pocket spending and unpaid caregiving [Alz. Assoc. 2026]. Our composite household lands above that national average, which is what you would expect: this is a ten-year course with 3.5 years in a facility, and the national figure is an average across every course, including the short ones.

I want to be careful with one more national number, because you will see it quoted and I do not want you to mis-stack it. A 2026 USC Schaeffer Center analysis puts the *total* cost of dementia to the United States at **\$818 billion for 2026** — roughly \$143,500 per patient per year — but that model includes the dollar value of reduced quality of life and of unpaid care, which the lifetime-cost figure treats differently [USC Schaeffer 2026]. The two are built on different definitions. They are not additive, and I am not going to add them for effect.

And this is Alzheimer's. If the diagnosis is Parkinson's, the shape is different and the burden is comparable: an annual national cost of **\$82.2 billion** across the disease, based on 2024 data, including **\$8.3 billion in lost earnings by care partners** [Parkinson's Foundation, 2024 data]. Different disease, same architecture — most of the cost lands outside the medical system, on people who are not the patient.

Why the ledger is shaped like that

Look at the middle column rather than the right one, because the middle column is where the money actually is.

In years one through three, the household is buying *things*. Medication copays. A grab bar. A different bathroom. An attorney's time to get the powers of attorney signed while Bill can still sign them. It is a bad year financially, not a catastrophic one, and it is entirely survivable — which is exactly why nobody restructures anything. The first three years produce a false read on the whole decade.

In years four through six, the household starts buying *hours*. This is the phase change. Somewhere in there is a specific week where Cindy admits she cannot leave the house to go to the grocery store, and the household starts paying \$35 an hour for someone else to be in the room. Twenty hours a week is not a lot of coverage — it is four hours a weekday, one shift — and it costs \$36,400 a year. Every additional hour they need is another \$1,820 a year, forever. Care is the one purchase in American life with essentially no economies of scale.

In years seven through ten, the household is buying *a bed with staff attached*, and the meter reads \$96,000 to \$130,000 a year until the day it stops. That is when the portfolio conversation happens, and by then it is not a planning conversation. It is a liquidation sequence.

Three phases. Things, hours, then a bed. That is the arc, and the reason it does not get intercepted is that the alarming phase arrives fourth.

Where the money actually comes from

Here is the part that a man who reads statements will want, and that most treatments of this subject skip entirely: not what it costs, but which account it comes out of.

It does not come out of income. That is the first thing to understand. Social Security and whatever pension or annuity income the household has will cover a meaningful slice of ordinary retirement living, and essentially none of a \$96,000-a-year care bill on top of it. So the shortfall is funded from the portfolio, and it is funded on the facility's schedule, not the market's.

Which means Bill and Cindy become forced sellers. Every month, for years, in whatever conditions the market happens to be in that month. The sequence-of-returns risk that their advisor spent an hour explaining to them at sixty-two — the risk that a bad early decade of withdrawals permanently impairs a portfolio — arrives at seventy-four, at a withdrawal rate nobody modeled, with no ability to defer.

Then the tax layer. Liquidating a taxable account to pay for care realizes gains. Drawing from a traditional IRA to pay for care is ordinary income, in a year when they have no offsetting

deductions to speak of, which can push them into a higher bracket and, at these levels, into higher Medicare premium surcharges. The bill has a bill attached to it.

Then the house, which is usually the last thing to go and the hardest. It is illiquid, it is where she lives, and the decision to sell it typically lands in year seven or eight — at the exact moment Cindy has the least capacity to run a sale.

None of that shows up in the right-hand column of the ledger. All of it is real, and all of it is what turns a \$500,000 care cost into something considerably worse on the balance sheet than a \$500,000 line would suggest.

The three phases produce the invoice. The liquidation sequence is what the invoice does to everything else.

The tail on the right

One caution about the ledger, and it cuts in the direction you will not like.

Bill and Cindy are a median household in a median market with a median course of disease. Averages are useful for sizing a category and dangerous for planning a household, because this particular distribution is not symmetric. There is very little room below the average and a great deal of room above it.

The downside cases are bounded. A short course — two or three years, most of it at home, death from something else first

— costs a household a fraction of the table above. That happens, and it is why the national average lifetime figure of \$405,262 sits below our ten-year composite.

The upside cases are not bounded in any comparable way. Early onset extends the course. Younger, physically robust patients live longer with the diagnosis, which is the cruelest arithmetic in this entire subject: the fitter the body, the longer the decade. A course that runs twelve or fourteen years instead of eight, with five of those years in a private room at \$129,575 a year in a high-cost metro, does not produce a \$560,000 bill. It produces something well north of \$800,000, and there is no ceiling written anywhere.

That asymmetry is the actual case for treating this as a catastrophic risk rather than a large expense. It is the same reason you carry an umbrella policy instead of self-insuring the liability — not because the expected loss is unaffordable, but because the tail is.

And the tail is exactly what has no carrier behind it.

The one that isn't on the statement

Every figure above is money that left an account. Somebody could audit it.

Now here is the part of this decade that no statement shows.

In 2025, roughly 13 million Americans provided more than **19 billion hours** of unpaid dementia care, valued at over **\$446 billion** [Alz. Assoc. 2026]. Divide that back down to one household and it is a person working an unpaid second full-time job for a decade — and unlike a second job, it is on call at three in the morning, and there is no version where she gives notice.

That labor never appears on the ledger, because there is no invoice for it. If you priced Cindy's hours at the same \$35 an hour the agency charges, the ten-year number in the table above roughly doubles.

This is the point at which most books about this subject would move on to solutions, having established the financial case.

I am not going to do that, because the financial case is not the case. The largest cost in this decade is not on the ledger and it is not paid by Bill.

Chapter 5 — The Second Patient

Cindy is seventy-nine when it ends.

She has been the primary caregiver for ten years, the last three of them driving to a facility every day because she could not stand the thought of him being there without a familiar face in the afternoon. She is the one who learned to give the medications. She is the one who took the car keys, which was the worst day of the marriage. She is the one who was told, at a family dinner, by a well-meaning son, that she looked tired.

And she is the second patient in this story. She was never diagnosed with anything. She was never entered into a chart. Nobody ever wrote her a treatment plan.

If you read only one chapter of this book, read this one, because everything that follows is built on it.

What the research says happens to her

A landmark 1999 JAMA study — Schulz and Beach, the Caregiver Health Effects Study — followed elderly spousal caregivers and found that those reporting caregiver strain had a **63% higher four-year mortality rate** than non-caregiving spouses of the same age [Schulz & Beach, JAMA 1999].

That study is twenty-seven years old. I am telling you its age on purpose, because I said in the front matter that I would. It remains the foundational, most-cited finding on this question, and nothing in the intervening decades has overturned it. But it is not new research, and you should hold it as what it is: a robust, old, still-standing finding that the strain of caregiving is associated with dying sooner.

Sit with the plain-English version of that sentence, though, because it is the whole chapter.

The diagnosis does not take one life. It taxes two.

The rest of the picture is consistent. Family caregivers report chronic conditions — heart disease, cancer, diabetes, arthritis — at nearly twice the rate of non-caregivers, **45% versus 24%** [Family Caregiver Alliance, undated; treat as an older figure]. Roughly **40% of dementia caregivers** show clinically significant depressive symptoms, against 5 to 17% of non-caregivers [Family Caregiver Alliance]. That is not sadness at a

hard situation. That is a rate two to eight times the background.

And they stop taking care of themselves, which is the mechanism connecting all of the above. The clearest data we have on that is from the Parkinson's world, where **34% of care partners canceled or missed their own healthcare visits** and more than one in five retired early or cut their work hours [Parkinson's Foundation, 2024 data].

That is how a healthy woman of sixty-nine becomes an unhealthy woman of seventy-nine. Not through one event. Through a hundred deferred ones. The mammogram she postponed twice. The cardiology follow-up she rescheduled and then forgot. The four years without a strength-training session because there was no window in the day that did not require someone else to be in the house. The sleep — the fragmented, listening-for-him sleep — which is its own slow injury and which she will get every night for a decade.

The decade, from her side of it

Let me put that research back into a house, because a percentage does not tell you what a Tuesday is like.

Year two, Cindy is still calling it *helping*. She has taken over the bills, which is not a hardship — she ran the books at the contracting company for eleven years before the kids. She has started driving to the club because Bill got turned around once,

and she has framed it as wanting the time together. She goes to her book club. She still plays.

Year four, the calendar on the refrigerator has three colors of ink on it and none of them are hers. There is a laminated card by the door with the neighbors' numbers. She has stopped going to the second half of things — she goes to the wedding and skips the reception, goes to the church service and skips the coffee after, because leaving him for three hours is a project and leaving him for six is not possible. This is the year the invitations start to thin out, not because anyone decided anything, but because she has said no eleven times.

Year six, she is not sleeping. He is up at two, and then at four, and she is listening for him even in the hours he is down. She has a cardiology follow-up on the calendar that she moves twice and then lets go. The aide comes four hours a weekday, which is enough to get to the grocery store and the pharmacy and back, and is not enough to get to a gym, and she has not lifted anything heavier than a laundry basket in four years. Her daughter, on the phone from Dallas, says *Mom, you have to take care of yourself*, and Cindy says *I know*, which is true, and does nothing about it, which is also true.

Year eight, he is in the facility and she is there every afternoon, and she has come to understand something she cannot say to her children: that the hardest part of this was never the money. It was the year she was grieving a man who was still sitting at the table.

That is what a 63% mortality figure is on the inside. It is not a dramatic event. It is a decade of a person systematically ranking herself last, in an environment where every single one of those individual decisions was the right call.

What it costs her that the numbers do not reach

Now put the money back next to it, because this is a book about money and I promised to keep it there.

Cindy is seventy-nine. Roughly half a million dollars of the retirement she and Bill built together is gone, spent down over a decade in the pattern of Chapter 4. If the spend-down went all the way to Medicaid eligibility, she is living inside a resource allowance and a protected monthly income figure set by a state formula.

She has ten or fifteen years left, if the mortality research does not apply to her. She is entering them with a fraction of the assets, a body that got no maintenance for a decade, and a social network that quietly thinned out around year six the way social networks do.

And here is the sentence I would put on the wall of every dinner talk I give:

She did not get the diagnosis, and she is paying for it longer than he did.

Bill's decade ends. Hers keeps going.

Who this book is actually for

I said at the front that this book is written for the person who reads the statements, and that it is usually a man. Here is the reason I hold that seat and keep speaking to it.

If you are that person, you have spent thirty years buying down risk on her behalf. That is what the umbrella policy was. That is what the term life was — you were not protecting yourself with it, you were never going to see a dollar of it. You bought it so that the worst version of your absence did not land on her as a bill.

The uninsured decade is the one exposure where that instinct has nowhere to go. There is no carrier to hand it to. And if it goes unaddressed, the outcome is the precise thing you spent three decades insuring against: the worst version of what happens to you, landing on her — not as a check she cashes, but as a decade she spends.

That is the whole argument of this book and I have now made it. Everything after this is arithmetic and logistics.

I am not going to tell you that you can prevent this. I said that in Chapter 1 and I will say it again in Chapter 9 with the evidence laid out, because it is the sentence that separates honest people from the other kind. What I am going to tell you is that this is a probability, that probabilities can be moved, that moving this one is the only reserve you can still post against the only risk nobody will underwrite for you — and that the person on the other side of that trade is not you.

Turn the page and we will price it.

Part II — The Premium

Chapter 6 — The Dividend

Every policy in that folder has one thing in common, and it is the thing nobody says out loud at the closing table.

You hope it never pays.

That is the arrangement. You write a check every year for thirty years, and the best possible outcome — the one you are actively rooting for — is that the money is gone and nothing came back. The umbrella policy that never got touched was a success. The term life that expired unclaimed was a triumph. You paid for the absence of an event. That is a strange product when you look straight at it, and it is the only shape insurance can take.

Now I am going to describe a policy with a different shape, and I want you to notice how uncomfortable it makes you, because a thing that sounds better than insurance sounds like a thing that is being sold to you.

The premium in this book has two payouts.

The first payout is the one you have been reading about for five chapters. It is contingent. It arrives, if it arrives at all, as a decade that did not happen — a ledger that never got written, an aide who was never hired, a Tuesday afternoon that Cindy spent at her book club instead of in a parking garage. Like every other policy in your folder, you will never be able to prove you collected it. You will only be able to notice, at eighty-two, that the invoice never came.

The second payout is not contingent, and it does not wait.

It starts inside about six weeks. It is paid in sleep that goes unbroken, in the fact that you can pick up a grandchild without doing the calculation first, in an afternoon at four o'clock that does not require a decision about coffee. It is paid in a waistband, a lab report, an erection, a walk up a jetway with a bag over your shoulder. It is paid in showing up to your own life at a temperature you had quietly written off as something that belonged to a younger man.

You do not have to develop dementia to collect it. You do not have to *avoid* dementia to collect it. It pays on the premium itself.

It is the only policy in the folder that pays before the claim.

I have thought for a long time about how to say this next part without it turning into the thing I told you in Chapter 1 that I would not do to you.

What you are actually buying

You did not pick this book up because you wanted to feel better. You picked it up — or somebody handed it to you and said *read the first six pages* — because of a number. The number is real, and I have spent four chapters making sure you cannot un-see it.

But fear is a starter motor. It is not an engine. I have watched a great many capable people get scared into a decision in a room and then not do the thing, and it is never because they stopped believing the numbers. It is because at six-fifteen on a Tuesday morning in March, a thing you are doing to avoid a statistic in 2044 loses to a warm bed every single time.

What does not lose to a warm bed is a thing that is already paying you.

So the second payout is not a bonus I am tacking on to make the financial case more attractive. It is the mechanism by which the financial case actually gets executed, which is a different claim and a more useful one. The dividend is what makes the premium payable for twenty years instead of eleven weeks.

And it is the part of this that I actually care about, so let me say it once, plainly, and then get back to the arithmetic.

The goal is not to not get dementia. The goal is not even to protect the money, though the money is why we are talking. The goal is the best mind you can have until your last day of life —

the whole span of it, not the tail. Sharp at seventy-eight. Present at eighty-four. Still the one people call when something has to be decided.

That is what the dividend is. Everything in this book is denominated in it.

Healthspan, in the language you already use

If you have run a business you already have the vocabulary for this, and it is not a medical one. There is a difference between the life of an asset and its *useful* life. You know this because you have depreciated things. The truck runs for nineteen years; the truck is worth having for eleven. The gap between those two numbers is not a rounding error. It is the whole question.

The medical system measures the first number and is very good at it. The second number is the one you live in, and the gap between them has been widening for forty years. We got very good at extending the span and much less good at extending the useful part of it — which means the marginal decade we added is, for a great many people, the decade in Chapter 4.

You do not want the long life. You want the long *useful* life, with the decline compressed into as short a window as the body will allow, at the far end, where it belongs. The research literature calls that healthspan, as against lifespan. Hold it in

the language of the depreciation schedule instead, because that is the language in which you actually make decisions.

The dividend, itemized

Let me be specific, because “quality of life” is the emptiest phrase in this entire subject and I do not want to hand it to you.

Sleep. Not eight hours as a virtue — seven or eight hours that are *consolidated*, so you wake at six and the day starts, rather than waking at six having been up at one-forty and again at four. This is the first thing that changes and usually the one people notice.

Strength. The kind that shows up as capacity: two suitcases up a flight of stairs, getting off the floor without a hand on the coffee table, a golf swing in October that is the same swing it was in April. Muscle is not a vanity account. It is the reserve you draw on during every bad week of the next thirty years, and after about fifty you are either adding to it or spending it down.

Energy at four in the afternoon. The most underrated marker at your age, and the one your spouse notices before you do. The four-o’clock collapse is not aging. It is usually blood sugar, sleep debt, and a lunch that was mostly bread.

Sex. Part III spends a chapter here, because it is not a lifestyle amenity — it is the vascular system filing an early report, and it is the most reliable canary either partner in a marriage gets. It is also, plainly, a payout.

Presence. The hardest to quantify and the one people actually mean. Being genuinely in the room. Following a complicated conversation at the end of a long day. Interested rather than depleted. Your grandchildren will not remember whether you were fit. They will remember whether you were *there* — and “there” is a metabolic state before it is a character trait.

The active retirement decade. Sixty-eight to seventy-eight is the decade you were saving for, and most people spend it at a level of function they never chose and never noticed choosing. The dividend is that decade arriving at the capacity you planned it at.

Read that list again and notice something about it. Not one line on it requires a diagnosis, a scare, or a statistic. Every line pays out inside a year.

The Premium

The premium: roughly ten hours a month and roughly eight to ten thousand dollars a year, most of it money you are already spending. Chapters 7 and 8 price it out line by line.

What it buys: two payouts from one premium. A contingent one — lower odds on the decade in Chapter 4, stated as odds and never as a promise. And a non-contingent one, paid out while you are alive to spend it: sleep, strength, energy, sex, presence, an active retirement decade.

What it does not buy: immunity. Nothing in this book prevents, cures, or reverses dementia. Anyone who tells you otherwise is selling something, and Chapter 9 is where I show you the math with nothing hidden.

That box is going to keep showing up. Same three lines every time. It is there so that the honest part never gets relegated to a disclaimer at the back.

What a dividend would have looked like for Bill

Bill was fifty-five once, and this is the part of his story I have not told you.

At fifty-five he was six years from selling the business and working the hours that implies. He weighed about thirty pounds more than he had at forty. His A1c had come back at 5.9 that spring and the nurse had used the word *borderline*, which he heard as *fine*. He slept badly, had for years, and thought of it as a personality trait. He had stopped playing tennis for three or four years in there because of a knee he never got looked at.

Nobody handed Bill a book at fifty-five. If somebody had, and if it had opened with the ledger from Chapter 4, I am not sure it would have worked, and I want to be honest with you about that. Fifty-five-year-old Bill would have read four chapters about a bill arriving in 2039, agreed with every word, and put it in the pile on the corner of the desk. He was not an unserious man. He was a man with a Thursday.

What might have reached him is the other payout. Not *this lowers your odds of memory care at seventy-six* — true, and fifteen years past the horizon he was operating on. Something closer to: *you will sleep through the night, the knee is a strength problem and not a knee problem, you will get the four-o'clock back, and Cindy will get her Saturdays back, starting in about two months.*

Run that forward. Bill at sixty-two, at the weight he was at forty-five, A1c at 5.3, sleeping. Bill at sixty-eight selling the business into an active decade instead of a tired one. Cindy at sixty-nine going to Portugal instead of learning to manage medications. Nobody can tell you what that does to the diagnosis at seventy — maybe it moves it, maybe it does not, and Chapter 9 is where I stop hand-waving and show you exactly what the evidence supports.

But this I can tell you with no statistical hedging at all: the thirteen years between fifty-five and sixty-eight would have been materially better years. Those years are not contingent on anything.

That is the dividend. It is paid whether or not the claim is ever filed.

And it is why the line on the cover of my first book belongs in this one too. Don't lose your identity and your dignity while you still have a choice. The ledger is about the money. The dividend is about the choice.

Which leaves the only two questions a person who reads statements actually cares about. What does it cost in hours, and what does it cost in dollars. Starting with the hours, because that is the one you think you do not have.

Chapter 7 — Ten Hours a Month

Ask someone in their fifties why they are not doing this and the answer is never the money.

It is time. And they mean it — that is not an excuse, it is a genuine constraint, and if you asked me to produce a spare four hours a week out of a calendar that is already full I could not do it either.

So let's find out what the actual number is, because in my experience it is not the number in his head. The number in his head was set by a magazine article in 1998 and it involves an hour a day, six days a week, plus a commute to a facility.

Here is the real schedule.

The line items

Two strength sessions a week. Forty-five minutes each.

Not five. Two. Compound movements — a push, a pull, a hinge, a squat, a carry — loaded heavily enough that the last two repetitions are genuinely difficult, which is the entire active ingredient and the part almost everybody skips. Two sessions a week done seriously beats five sessions a week done conversationally, and it is not close.

Forty-five minutes includes the warm-up. It does not include a sauna, a smoothie, or a conversation at the front desk. If you are in and out in an hour door to door, you are running it correctly.

Ninety minutes a week.

The fasted morning walk. Twenty minutes, five mornings.

Before you eat. That is the whole specification, and the reason for it is Chapter 11's business — but the short version is that a walk taken in a fasted state does something metabolically different from the same walk taken after breakfast, and it is the single most accessible lever most people have.

Twenty minutes. Not a workout. Outside if you can, for the light, which is doing more than you think. This is also where the protein rule attaches, and I will state it here in one line so it is on the record early: **you break the fast after the walk, and you break it with thirty to forty grams of lean protein before anything else goes in.** Not with the coffee-and-toast that has been the default since 1974.

A hundred minutes a week.

Cooking. Three or four hours a week.

This is the biggest line item and it is the one that is not what it looks like, which is why I have put it third instead of hiding it.

Fifteen minutes a week of planning. One sitting, usually Sunday. What is on the calendar, what is getting cooked, where the two strength sessions actually go — written down, in the same calendar you use for everything else that gets done.

The gross number and the net number

Do the arithmetic honestly and the gross number is not ten. The strength sessions are six and a half hours a month. The walks are another seven. The planning hour is one. That is roughly fifteen hours a month before you count a single minute of cooking — and cooking is three or four hours a week on top of it. I am not going to shave that to make the chapter title work.

The chapter title works because of what those hours *displace*, so let's do that subtraction in the open.

The cooking is essentially a wash. You are already eating. The relevant comparison is not cooking versus nothing — it is cooking versus the drive-through on the way home, the twenty-two minutes waiting for a table, the delivery order and the

delivery order's aftermath. A household that cooks four dinners a week instead of outsourcing them is not spending three or four net-new hours. On a full accounting — the drive, the wait, the ordering, the cleanup either way — it is close to zero, and it gets the dinner table back, which is not a health outcome but is not nothing.

The walk is mostly a wash. For most people the twenty minutes before breakfast currently goes to a phone, to a news cycle that will be identical at eight o'clock, or to the specific low-grade activity of not quite being awake yet. Call a third of it genuinely new time — about two and a half hours a month. The rest is a reallocation of minutes you were already awake for.

The strength sessions are not a wash. Ninety minutes a week, six and a half hours a month, and every minute of it is new. That is the real cost and I am not going to dress it up. Neither is the planning hour, which is another one.

Add them: six and a half, plus two and a half, plus one, plus roughly nothing for the cooking. Net new time, honestly counted: **about ten hours a month. Two and a half hours a week.**

Now put that against the rest of the calendar. Ten hours a month is:

- About two percent of your waking hours.

- Less time than the average American adult your age spends watching television in three days.
- Less than the drive to a single out-of-town grandchild's weekend.
- Roughly one Saturday morning a month, spread out.

If your calendar cannot absorb ten hours a month, the calendar is the finding, not the protocol.

What it actually displaces, which is the honest part

I want to be careful not to sell you a free lunch, because there is a version of this chapter that says “you already have the time!” and it is not quite true.

Here is what usually gets displaced in a real week, in the order I see it happen:

Television, first and mostly. This is not a moral point. It is just where the hours are. The average American in your age bracket is running three-plus hours a day here, and the strength sessions come out of it almost invisibly.

The unstructured evening hour. The one between finishing dinner and deciding to go to bed, which historically has been the hour in which a second drink happens. That hour tends to reorganize itself, and Chapter 10 has a lot to say about why that particular displacement has a compounding return.

Some morning slack. Twenty minutes earlier, five days. This is the one that costs something real for the first three weeks and then stops costing anything, because it moves your sleep timing with it.

Occasionally, one work thing. Sometimes the seven a.m. call is the thing that has to move. If you have built a life in which nothing can ever move for you, that is worth noticing on its own terms, and it is the same finding as the calendar one.

Nothing on that displacement list is a sacrifice you will be able to name at eighty. Nobody has ever reached the end of the useful decade wishing they had gotten more evening television in.

Why this is the cheapest line item in the plan

Now put the hours next to everything else you have ever done to manage a risk of this size.

Getting the umbrella policy right took a phone call and a signature — cheap, and it covers a risk you will probably never have. Getting the estate plan right took two meetings with an attorney and roughly what those meetings cost. Getting the business sold took eighteen months and most of what you had.

Getting your physiology onto a different trajectory takes two and a half hours a week, and it is aimed at the largest uncovered exposure on the schedule.

I do not know of another line item in a competent household's risk plan with that ratio. Not one.

And there is a compounding property in the hours that is not in the dollars. The dollars in the next chapter are a flat annual number — you pay it, it is gone, you pay it again. The hours are not flat. Ninety minutes a week of loaded strength work at fifty-eight builds something that is still on the balance sheet at seventy-two. You are not renting the muscle. You are building an asset and then maintaining it at a fraction of what it cost to build.

That is the closest thing to compound interest that exists in a body, and you are running out of years in which the compounding period is long enough to matter. Not out of time. Out of *compounding* time, which is a different and more urgent scarcity, and which is the only piece of genuine urgency in this entire book.

The Premium

The premium: *ten net-new hours a month. Two forty-five-minute strength sessions a week, a twenty-minute fasted walk five mornings, four dinners cooked instead of ordered, fifteen minutes of planning on a Sunday.*

What it buys: *the dividend, starting in about six weeks — and the only intervention with randomized-trial evidence behind it on the cognitive side, which Chapter 9 lays out with the numbers showing.*

What it does not buy: *a guarantee, and not a single one of those hours can be banked, borrowed against, or paid in a lump sum at sixty-nine. It is a premium. It is due monthly.*

Now the money, which is the part you thought was going to be the problem.

Chapter 8 — Pricing the Premium

I am going to give you the whole basket, at three price points, with sources.

Then I am going to do the thing almost nobody does with a number like this, which is to separate what is genuinely new money from what is already leaving your account under a different label.

The basket

These are national ranges for 2025 and 2026. Your market will vary and you should price your own.

Line item	Low	Mid	High
Gym membership, mid-tier	\$30/mo	\$60/mo	\$120/mo
Personal trainer, once a week	\$160/mo	\$280/mo	\$560/mo
Grocery premium — better protein and produce, two-person household	\$120/mo	\$185/mo	\$250/mo
Supplement stack	\$100/mo	\$150/mo	\$200/mo
Lifestyle subtotal	\$410/mo	\$675/mo	\$1,130/mo
Annualized	~\$4,900/yr	~\$8,100/yr	~\$13,600/yr
<i>Optional: one compounded prescription therapy where indicated and prescribed</i>	\$150/mo	\$225/mo	\$350/mo
Annualized, with one prescription therapy	~\$6,700/yr	~\$10,800/yr	~\$17,800/yr

[premium basket pricing, 2025–2026]

A few notes on how to read that table honestly.

The trainer line is optional and it is the largest single item. Drop it entirely and the mid column falls from about \$8,100 to about \$4,700 a year. I keep it in the default basket because two heavy compound sessions a week done with bad mechanics is how people in their sixties get hurt and quit, and

eight to twelve weeks of a competent coach at the front end is the cheapest injury insurance in the basket. After that it is genuinely optional, and plenty of people go to once a month or drop it.

The grocery line is a delta, not a total. It is the premium over what you are already spending on food, not the food budget. If your household is currently eating four restaurant dinners a week, this line is negative.

The prescription line is one item, not five. There are several categories where a licensed clinician may or may not determine that prescription support is appropriate for a given person — compounded prescription support for gut-barrier repair where indicated, weight-management medication where indicated, hormone support where indicated, regenerative therapies for those already carrying a diagnosis. A household typically uses none of these or one of them. Nobody sensible is on all of them, and I have not stacked them in the table. Which, if any, applies to you is a determination made by an independent licensed prescriber after labs and an actual clinical evaluation, and it is not something this book can tell you.

So: **roughly \$8,000 to \$10,000 a year for the lifestyle-only version**, and **roughly \$15,000 to \$18,000 a year if a prescription therapy is part of it.** Those are the numbers I will use for the rest of the book.

Now subtract what you are already spending

Here is where the number gets less alarming, and I want you to do this on your own statement rather than take my word for it.

Basket line	What you are probably already paying	Genuinely new money
Gym membership	Most households in this income band already carry one — frequently one that is not being used, which is a different problem	\$0 to \$60/mo
Trainer	Usually new	\$160–\$560/mo, and optional after the first three months
Groceries	You are buying food every week regardless; this line is only the quality delta, and it drops as restaurant meals drop	\$0 to \$185/mo, often less
Supplements	Most men over fifty already have a shelf of these, bought without a plan and mostly not being taken	\$0 to \$100/mo net
Prescription therapy, if indicated	New	\$150–\$350/mo

Run the mid column with that subtraction applied to a typical household and the genuinely new money is somewhere between three and five thousand dollars a year, not eight. The rest is reallocation — the same dollars, aimed.

That is not a trick and it is not softening. It is the single most common thing I see when a household actually itemizes this: they are already spending most of the premium. They are spending it on an unused membership, a cabinet of bottles nobody chose deliberately, and a food budget with a restaurant line in it that nobody has looked at in three years. The money is not missing. It is unmanaged.

The premium against the face value

Now put it in the vocabulary you actually use.

Term life is priced as a premium against a face value. You do not evaluate a twenty-year, two-million-dollar policy by asking whether \$2,400 a year is a lot of money to spend on nothing. You evaluate the ratio.

So do it here.

- **Premium:** roughly \$10,000 a year.
- **Ten-year premium:** \$100,000.
- **Exposure:** the ten-year ledger in Chapter 4 — roughly \$495,000 to \$560,000 for a median household, with a

right tail past \$800,000 and no ceiling written anywhere.

Call it a hundred thousand against a half-million-plus. Roughly a five-to-one ratio at the median exposure and considerably better than that in the tail — which is exactly the region insurance exists to address.

I want to flag something before you get comfortable with that ratio, because it is not the same ratio as your term policy and I am not going to let it pass as though it were.

Your term policy pays the full face value if the event occurs. **This premium does not.** It does not pay \$500,000 on a diagnosis. It does not pay anything on a diagnosis. What it does is reduce the probability that the event occurs at all — by some amount, which is a real amount with real evidence behind it, and which is smaller than the number you would like it to be.

That distinction is the entire subject of the next chapter, and it is where I show you the arithmetic and let it land wherever it lands.

The comparison you were about to make anyway

One more before we do the math, because you have been running it in the back of your head since Chapter 3.

A healthy sixty-year-old couple buying moderate long-term-care coverage today — around \$165,000 of initial benefit with

3% growth — is quoted somewhere in the range of \$4,600 to \$7,200 a year combined [industry pricing data, July 2026]. That is a comparable order of magnitude to the lifestyle basket above.

So the fair comparison is not “premium versus nothing.” It is: for roughly the same annual outlay, one product is medically underwritten and may decline you, is guaranteed renewable rather than premium-guaranteed and has a documented history of repricing on the people who bought it, caps out well below the ten-year exposure, and pays nothing at all unless you become impaired.

The other one cannot be taken away from you, cannot be repriced by a state filing, is available regardless of your health history, and pays a dividend every year whether or not the claim is ever filed.

I am not telling you to buy one instead of the other. I do not sell either one, I cannot advise you on the first, and some households should own both. I am telling you that if you have already decided the first one is too expensive for what it delivers, you should apply the same analysis to the second one and see where it comes out.

The Premium

The premium: roughly \$8,000–\$10,000 a year for the lifestyle basket; roughly \$15,000–\$18,000 if a prescription therapy is indicated and prescribed. Genuinely new money, for most households: \$3,000–\$5,000 a year. The rest is money you are already spending, aimed.

What it buys: a reduction in probability — not a benefit payment — on a \$495,000–\$560,000 median exposure with an unbounded right tail. Plus the dividend, which pays annually and unconditionally.

What it does not buy: a face value. There is no check on diagnosis. If you are looking for a product that pays out when the event occurs, that product is long-term-care insurance, and Chapter 3 is where I told you what happened to it.

Now the arithmetic, with nothing rounded in my favor.

Chapter 9 — Odds, Stated Honestly

I promised in Chapter 1 that I would not tell you this pays for itself.

Here is where I keep that promise, with the arithmetic on the page, and I would rather lose you here on an honest number than keep you on a bent one.

The assumptions, stated before the math

Four inputs. Argue with any of them and the answer moves; that is the point of listing them.

One. Lifetime risk of dementia after fifty-five: 42%.

From the ARIC cohort study published in *Nature Medicine* in January 2025, following more than 15,000 people from age fifty-five — 42% overall, 35% for men, 48% for women [Fang et al., Nat. Med. 2025].

Two. For a mixed-sex couple, the probability that at least one of you develops it: about 66%. That is not a

separate finding. It is arithmetic on the first one: $1 - (0.65 \times 0.52) = 0.662$.

Three. Average lifetime cost of care if it happens: \$405,262, in 2024 dollars, roughly 70% of it borne by the family [Alz. Assoc. 2026].

Four. Risk reduction from sustained structured lifestyle change: 30%. This is the number I have to defend hardest, and I defend it below. It is a conservative reading of the evidence and it is deliberately lower than the figure you have probably seen quoted.

Before I multiply anything, one caveat that has to travel with the 42% every single time it is used.

That 42% is all-cause dementia, including mild late-life cases. It is a broader category than “Alzheimer’s requiring years of full-time memory care,” and a meaningful share of the people inside it will experience something considerably less severe and less expensive than the decade in Chapter 4. The \$405,262 figure, meanwhile, is the average cost among people who develop dementia and require care — not further discounted for the mild cases.

So do not read the table below as “66% of couples face a half-million-dollar decade.” That is not what these numbers say. Read it as what it is: a risk-weighted expected value, using the broadest risk figure and an unadjusted average cost, which pull in two directions at once.

The math

	Lifetime risk	Expected cost, unmitigated (risk × \$405,262)	Expected savings at 30% risk reduction	Net against a \$100,000 ten-year premium
Single person	42%	\$170,210	\$51,063	-\$48,937
Couple — “does this happen to either of us”	66.2%	\$268,283	\$80,485	-\$19,515

There it is.

At the honest risk-reduction assumption, a couple paying \$10,000 a year for ten years comes out roughly **nineteen thousand five hundred dollars short** on dementia-cost avoidance alone. A single person comes out about **forty-nine thousand short**.

It does not break even. I told you in Chapter 1 that it would not and I am not going to pretend now that it did.

The number you have seen quoted, and why it is not in that table

You have probably encountered a much friendlier figure: that up to 45% of dementia cases are attributable to modifiable risk factors. It comes from the 2024 Lancet Commission, which identified fourteen of them — hearing loss, hypertension, smoking, obesity, depression, physical inactivity, diabetes, excess alcohol, head injury, air pollution, social isolation, high LDL, untreated vision loss, and low education [Lancet Commission 2024].

That is a real finding from serious people, and it is not what most people who quote it think it is.

45% is a **population-attributable fraction**: the answer to the question *if every person on earth eliminated all fourteen of those factors, across the whole life course starting in childhood, what share of dementia cases would theoretically not occur?* It is a public-health ceiling, describing a population over generations under conditions that will never obtain.

It is not a statement that you, personally, starting at fifty-eight, cut your own risk by 45%. No study shows that. Anyone who hands you that number as a personal figure either does not understand the distinction or is counting on you not to.

For the record, here is where the ceiling would put the couple's arithmetic: $\$268,283 \times 0.45 = \textbf{\$120,728}$, which is \$20,728

above the \$100,000 premium. I show it so you know I did not hide it, and I label it plainly: **a population ceiling under impossible conditions, not a result any individual household should expect.** It is the only version of this arithmetic that produces a positive number, and I am not going to sell you the positive one.

30% stays the working assumption.

What the individual-level evidence actually shows

Since I am asking you to accept 30%, here is the evidence underneath it.

The strongest single piece is US POINTER, published in *JAMA* in July 2025 — 2,111 participants, mean age 68.2, two years, randomized. Both a structured multidomain lifestyle program and a self-guided version improved global cognition, and the structured arm improved significantly more than the self-guided one. The benefit held across age, sex, ethnicity, cardiovascular status, and APOE-ε4 genotype [US POINTER, *JAMA* 2025]. It built on the Finnish FINGER trial, which showed the same direction over two years a decade earlier [Ngandu et al., *Lancet* 2015].

Read what that says and also what it does not. It is an improvement-in-cognition and protection-from-decline finding over two years. It is **not** a finding that anybody prevented de-

mentia. No trial has run long enough to show that, and when someone cites US POINTER as proof that lifestyle prevents Alzheimer's, they have read the headline and not the paper.

Physical activity, pooled across meta-analyses: relative risk for all-cause dementia of about **0.80** in the more active — roughly a 20% lower risk. Alzheimer's-specific, about 0.86. Vascular dementia, about 0.79 [Iso-Markku et al., BJSM 2022]. Leisure-time activity specifically, about 0.76 [Feter et al., Lancet Public Health 2026]. One caution that tells you these are real analyses and not marketing: occupational physical activity showed a slightly *increased* risk in one large pooled analysis. The benefit is domain-specific. Being on your feet all day at work is not the same exposure as training.

That is roughly 20 to 24% from one domain, observational, with all the confounding that implies. Stack a second and third domain — metabolic control, sleep, hearing, blood pressure — and 30% is a defensible aggregate. It is also, I want to be clear, an *estimate assembled across evidence types*, not a number that any single trial has produced for a multidomain program. That is the honest state of the field.

Three things that tip the ledger

So the dementia-only arithmetic comes up short. Here is why I still think the premium is the right trade, and each of these is a real argument rather than a consolation.

One: you are not buying one risk reduction. You are buying several, simultaneously.

This is the largest of the three and the one the expected-value table structurally cannot capture, because it prices exactly one line item.

The strength work, the metabolic control, the sleep, and the removal of chronic inflammatory load do not act on a dementia pathway and nothing else. They act on the same substrate that produces type 2 diabetes, hypertension, cardiovascular disease, stroke, and the sarcopenia that decides whether a fall at seventy-eight is an inconvenience or the beginning of the end.

And those conditions are not sitting in a separate column from dementia. They are the road that runs to it. The hazard ratios are specific and they are not small: a type 2 diabetes diagnosis at sixty to sixty-nine carries a **hazard ratio of 1.70** for subsequent dementia; diagnosed at fifty to fifty-nine, **1.72**; diagnosed before fifty, **1.90** — each measured against diagnosis at seventy or later. And obesity plus diabetes diagnosed before fifty carries a hazard ratio of **3.05** against non-obese diagnosis at fifty or later [Qi et al., PLOS ONE 2024].

Read that last one again. Roughly a tripling, from two conditions that are both largely addressed by the same ten hours a month.

So the premium is not buying a 30% reduction on one exposure. It is buying a partial reduction across a correlated portfolio of exposures, several of which are themselves inputs to the

exposure in the table. The expected-value math above prices one of those and ignores the rest, which means it is not a neutral estimate — **it is a floor**. The dementia case is the hardest case to make for these behaviors, not the easiest, and it still comes within nineteen thousand dollars of break-even.

Two: the right tail.

The \$405,262 in that table is an average. Chapter 4 told you what the distribution looks like around it, and it is not symmetric.

The low side is bounded — a short course, two or three years, mostly at home. The high side is not. A twelve- or fourteen-year course with five years in a private room in a high-cost metro does not produce a \$560,000 bill. It produces something well past \$800,000, with no ceiling written anywhere.

Expected-value arithmetic on an asymmetric distribution systematically understates the thing you are actually protecting against, because it averages a bounded downside against an unbounded upside and reports the middle. The middle is not the risk. The tail is the risk.

Which brings me to the sentence this whole chapter has been walking toward.

You do not insure the house because you expect it to burn.

You expect, quite confidently, that it will not. The expected value of your homeowner's policy is deeply negative and has been every year for thirty years, and you have never once considered dropping it, because you are not buying expected value. You are buying the elimination of an outcome you cannot come back from.

That is ruin avoidance, and it is a completely different trade from an investment. You have made it a dozen times without needing a spreadsheet to justify it. The umbrella policy has a terrible expected value. The term life had a terrible expected value. You bought them because the loss, if it landed, was unrecoverable — and unrecoverable is the test, not likely.

The uninsured decade is the last unrecoverable loss on your schedule, and it is the one where the reserve has to come out of physiology because no carrier will post it for you.

Three: the dividend, which the table cannot price at all.

Look at that expected-value calculation one more time and notice what it is doing. It is treating \$100,000 of premium as pure cost — money spent purchasing nothing but a probability shift. That is not what happened.

You bought ten years of sleeping through the night. Ten years of getting off the floor without a hand on the coffee table. Ten years of four o'clock in the afternoon not being a wall. Ten years of a marriage in which the physical part did not quietly

close down at sixty-four. Ten years of being present in rooms you would otherwise have been merely attending.

None of that is in the table. All of it was delivered, unconditionally, whether or not the claim ever gets filed.

If you charged even a modest annual value against those ten years — and you would pay considerably more than \$10,000 for a year of good sleep and functional strength if someone offered it as a product, which they cannot — the ledger closes and then some. I have not put a dollar figure on it because any figure I chose would be invented, and I have gotten this far on numbers you can check. But you can price your own, and you should, because it is the largest term in the equation and the one I left blank on purpose.

Where that leaves the trade

The whole thing in one paragraph, with nothing rounded in my favor. Paying roughly \$10,000 a year and ten hours a month does not, on the dementia-cost arithmetic alone, return more money than it costs — for a couple it comes about nineteen thousand five hundred dollars short over ten years at a conservative 30% risk reduction. Closer than most people expect. Not break-even. What tips it is that the same premium reduces a correlated set of other risks the table never priced, that the exposure has an unbounded right tail averages cannot represent, and that the premium delivers an unconditional annual dividend the table treats as worth zero.

If you need a positive expected value on a single line item before you will act, do not do this.

If you have ever bought a policy on a risk you did not expect to occur, because the version where it did was one your family could not come back from, then you already understand this trade. You have made it before. The only difference is that this time there is nobody to write the contract, and the reserve has to come out of hours.

The Premium

The premium: *ten hours a month. Roughly \$8,000–\$10,000 a year, most of it money already leaving your account.*

What it buys: *a conservative 30% reduction in the odds of the decade in Chapter 4 — which does not break even on that line item alone — plus a reduction across a correlated set of exposures the arithmetic never priced, protection against a tail with no ceiling, and a dividend paid annually and unconditionally starting in about six weeks.*

What it does not buy: *immunity, a benefit check, or a guarantee. Nothing in this book prevents, cures, treats, or reverses dementia, Alzheimer's, or Parkinson's. It lowers the odds. That is the only sentence an honest person can sell you, and it is the only one I will.*

You have the exposure. You have the price, in hours and in dollars. You have the odds, stated as odds.

So here is the coverage schedule.



Part III — The Policy

Chapter 10 — Mind: The Destination Has a Line Item

Every policy in that folder has a schedule of benefits. It is the page nobody reads — the one near the back, in the smaller type, that says exactly what is covered, up to what limit, subject to what conditions.

This one has a schedule too. It is four lines long.

Line item	What it covers	Hours a month	Dollars a month
Mind (Ch. 10)	Sleep, morning light, the gut barrier, and three front doors — hearing, vision, teeth	~0 net new	\$100–\$200 supplements; some one-time hardware; \$150–\$300 if prescription gut-barrier support is indicated
Muscle (Ch. 11)	Two strength sessions a week, the fasted morning walk, protein first, visceral fat	~9	\$190–\$680 gym and coaching; \$150–\$300 if weight-management medication is indicated
Mitigate (Ch. 12)	A1c, LDL, blood pressure, alcohol, smoking; hormones and the two canaries; air and water	~0 — this line is mostly subtraction	labs and consults; \$150–\$350 if hormone support is indicated
Motivate (Ch. 13)	One named face, one sentence about who you are, fifteen minutes on a Sunday	~1	\$0

Four lines. Ten hours. Most of the dollars already leaving your account under a different label, as Chapter 8 showed you on your own statement.

Each of the next four chapters takes one line and reads it the way you would read any other coverage: what it covers, what it

costs, what the evidence says it buys — stated as odds, the way I promised — and what the line looks like on the ledger if you leave it blank.

One structural note before the first one. In the framework these four lines come from, Mind is not a pillar sitting alongside the other three. Mind is the destination. Muscle, Mitigate, and Motivate are the three things that pay for it. That is what *begin with the end in mind* means on the cover of my other book, and it works three ways at once — start from the outcome and reason backward, the way you would with any plan; the outcome in question is literally your mind; and a voyage that is worth taking ends where it started, with the same man walking back through his own front door.

But a schedule of benefits still needs a line for the destination, because there is work that goes directly to the brain and nowhere else. That is this chapter. It is also, by a wide margin, the cheapest line on the page in hours, which is why I put it first.

What happens at night

Start with sleep, because it is the only line item on this entire schedule that you are already spending eight hours a day on and getting no return from.

Sleep is not rest. In 2013 a research team at the University of Rochester described a waste-clearance network in the brain —

cerebrospinal fluid moving through the spaces between brain cells and flushing out the metabolic debris of the day, including the protein fragments most directly implicated in Alzheimer's pathology [Xie et al., *Science* 2013]. The relevant feature of that system is that it is largely dormant while you are awake. It runs during deep, slow-wave sleep, in the first third of the night.

There is no supplement that substitutes for that window. There is no workout that substitutes for it. The trash goes out at night or it does not go out.

The second job of the night is filing. During REM sleep, the day you just lived gets moved from short-term storage into long-term storage. Miss it and the meeting from Tuesday is smudged at the edges by Thursday — which is, incidentally, the exact symptom that made you pick up this book.

So when someone your age tells me they sleep fine, the number of hours is not the interesting question. Four things are:

How long it takes to fall asleep. Five to twenty minutes is healthy. Under five means you are not falling asleep, you are passing out. Over thirty, consistently, means something upstream is wrong.

How many times you wake. Once around four is normal at your age. Three and four times is fragmentation, and fragmented sleep cannot deliver the consolidated blocks the two jobs above require. Six and a half consolidated hours beat eight broken ones.

How much of it is deep. Most people past fifty have quietly lost a large share of their deep sleep without ever knowing it. A wearable that reports sleep stages makes this visible for about \$300 one time, and it is the single most useful measurement in this book.

When it happens. Ten to five and midnight to seven are both seven hours and they are not the same seven hours. The hormonal work of the night is anchored to the clock, not to whenever you happen to lie down.

Now the displacement I promised you in Chapter 7. The evening hour that reorganizes itself when the strength sessions arrive is usually the hour in which a second drink happens, and that particular displacement has a compounding return, because alcohol is sedating and sedation is not sleep. It gets you down faster, then the suppression is dose-dependent and concentrates in the second half of the night, cutting into the REM block — the filing block — while you are unconscious and unable to notice [Ebrahim et al., *Alcohol Clin. Exp. Res.* 2013]. You wake at six-thirty having “slept eight hours,” and yesterday is already blurred.

That is not a moral point. It is an accounting point. You are paying for the drink twice, and the second payment is drawn from the account this whole book is about.

What your eyes see in the first

hour

The cheapest intervention on the entire schedule costs nothing and takes ten minutes, and it attaches to the morning walk you are already going to be taking in the next chapter.

Your body clock is set by light hitting the back of your eye in the first hour after waking. Outdoor light on an overcast morning is roughly ten to fifty times brighter than the lighting in your kitchen. Ten minutes outside, no sunglasses, inside an hour of waking, is the anchor. Everything downstream — when melatonin rises fourteen hours later, when cortisol peaks, when the deep-sleep window opens — is scheduled off that signal.

Then the other end of it. The overhead lights in your house and the screen in your hand emit heavily in the exact blue range your brain reads as *noon*. At nine at night you are telling a two-hundred-thousand-year-old system that the sun is directly overhead, and it responds the only way it knows how.

Amber bulbs in the rooms you use after dark. A red nightlight in the bathroom. The phone out of the bedroom. Call it a hundred dollars, once.

I am aware of how small that sounds next to a half-million-dollar exposure. It is small. It is also free, and free things that plausibly help are exactly the bets a person who prices risk for a living should be making.

What your gut is manufacturing

Now the part of this chapter that will be new to you, and the reason a book about money has a chapter about digestion in it.

Your gut is not only digesting food. It is manufacturing a large share of the neurochemistry your brain runs on.

About 90% of the body's serotonin — mood, sleep, appetite — is produced in the gut. Roughly half of your dopamine — motivation, reward, focus — is made there too. The microbial population living on the other side of that lining also produces the chemistry of calm and the chemistry of memory and learning. And the vagus nerve, the largest cranial nerve you own, is overwhelmingly a one-way street: roughly 80% of its traffic runs gut-to-brain, not brain-to-gut [Gershon & Tack, *Gastroenterology* 2007; Eisenhofer et al., *JCEM* 1997; Berthoud & Neuhuber, *Auton. Neurosci.* 2000].

Your gut is filing a continuous report to your head. The only question is what is in the report.

Here is the failure mode, in one line, because you do not need the biochemistry and I am not going to perform it for you. The lining of the gut is a single layer of cells — one cell thick — and it is the entire physical boundary between everything you eat and the rest of your bloodstream. Degrade that layer, and bacterial fragments cross into circulation that were never supposed to be there. The immune system reads that as an invasion and mounts a low-grade inflammatory response, every-

where, all the time. The same inflammatory traffic degrades the barrier protecting the brain. And the brain, which has no way to tell you what is wrong, translates it into the only vocabulary it has.

Fog. Flat mood on a perfectly fine Tuesday. The colleague's name that is gone for three full seconds. Reading the same paragraph three times.

You are not necessarily getting old. You may be inflamed. And the inflammation may be coming from a barrier breach happening every day in a tissue you have never once thought about.

What degrades it is not exotic: ultra-processed food as a daily baseline, industrial seed oils, chronic alcohol, repeated courses of antibiotics with nothing done afterward, and years of unmanaged stress. What repairs it is a specific protocol run for a specific window — not kombucha, and not the bottle of probiotics in your cabinet, most of which are strain-unspecific, unprotected against stomach acid, and mostly decorative by the time they arrive where they are needed.

For a household whose symptoms have been running for years, there is a prescription tier here — a compounded gut-barrier protocol, written by an independent licensed clinician to the individual after labs and an actual evaluation, and typically run for eight to twelve weeks on top of a daily over-the-counter foundation. I am not going to print the formula and I am not going to name a product, for the same reason your attorney

does not put his fee schedule in a book. Whether any of it applies to you is a clinical determination made by a prescriber, not a determination made by an author. What we do at my4m-life.com is put you in front of one.

Three front doors standing open

The last piece of this line item is the piece nobody spends anything on, and it is the piece with the best evidence-to-cost ratio in the entire book.

The 2024 Lancet Commission on dementia lists fourteen modifiable risk factors [Lancet Commission 2024]. You will recognize most of them — hypertension, smoking, obesity, diabetes, physical inactivity, excess alcohol. But three of them are not what anybody expects, and all three are things a competent household fixes in a month.

Hearing. Untreated hearing loss is on that list. Not hearing aids as a vanity concession — hearing loss as a load on the brain, which spends its reserve decoding speech instead of holding a conversation, and a household that withdraws socially because the restaurant is too loud. This is a hearing test and, if indicated, a device. Most insurance covers the test.

Vision. Untreated vision loss is on the list too. A cataract that has been getting quietly worse for six years is not just an eye problem. It is a light problem, which makes it a sleep problem,

and it is a reading and driving and social-engagement problem.

Teeth. This one surprises people. The bacteria of chronic gum disease have been identified in the brain tissue of Alzheimer's patients, and the leading periodontal pathogen is a specific named organism, not a vague association [Dominy et al., *Sci. Adv.* 2019]. Gum disease is systemic inflammation with an address. A periodontal probing once a year — not a cleaning, a probing — is the standard.

A hearing test, an eye exam, and a periodontal probing. Three appointments. Two of them probably covered. Nobody in your age bracket does all three, and every one of them is on the list of things the most serious dementia commission in the world says actually matters.

What the blank line looks like

Leave this line blank and nothing happens on Tuesday. That is the whole problem with it.

What happens instead is the slow version. Fragmented sleep that never gets diagnosed, so the overnight clearing window runs short for fifteen years. A body clock with no anchor, so the fatigue gets attributed to age and answered with caffeine. A gut barrier under daily insult, so the inflammatory load never comes down and the fog gets filed under *fifty-eight*. And three

front doors standing open the entire time, because nobody ever told you they were doors.

None of that produces an event. It produces a slope. And a slope that nobody intercepts is exactly the shape of the arc in Chapter 4 — where the invoice does not spike, it escalates on a schedule.

Why this line never closes

One honest thing about this line item, since I am asking you to fund it for the rest of your life.

The gut barrier turns over every three to five days. It is fast tissue: fast damage, fast repair. The barrier you are walking around with on Friday is not the barrier you had on Monday. That cuts both ways — it means there is no such thing as a finished gut, and it means improvement shows up in weeks rather than years.

Sleep architecture degrades with age on its own, without your help. The light your eyes actually receive declines as the lens yellows and as indoor life takes over. And every one of the insults above is delivered by ordinary modern life, continuously, whether or not you are paying attention.

This is not a project with a completion date. It is a line item. That is precisely why the insurance vocabulary is the right vocabulary — you do not “finish” a premium either.

The Premium

The premium: essentially no new hours — ten minutes of morning light that rides along with the walk in the next chapter, an evening hour that reorganizes itself, three appointments in the first month. In dollars: \$100–\$200 a month for a deliberate supplement foundation instead of an accidental one, a few hundred once for a sleep tracker and amber bulbs, and \$150–\$300 a month if an independent licensed clinician determines prescription gut-barrier support is appropriate for you.

What it buys: the overnight clearing and filing windows the brain has no substitute for; a body clock with an actual anchor; a lower inflammatory load reaching the brain; and three of the fourteen risk factors the Lancet Commission names, closed [Lancet Commission 2024]. In odds, not outcomes.

What it does not buy: immunity, and no claim that any of it prevents, treats, cures, or reverses anything. It lowers the odds. That remains the only sentence I will sell you.

That is the destination line. Now the largest one.



Chapter 11 — Muscle: The Reserve You Post in Advance

This is the expensive line. Nine of the ten hours are here, and most of the dollars.

It is also the line you already know how to run, which is why I am not going to spend the chapter motivating you. You have built things before. This is a building project with a completion schedule, and it is the only line on the page where the thing you buy stays bought.

What the line covers

Four items. Two strength sessions a week, a twenty-minute walk before you eat, protein first when you break the fast, and a waist measurement you take seriously.

That is it. There is no fifth thing.

Two sessions, and why they are the neuroprotective item

Start with the part almost nobody over fifty believes: lifting is a brain intervention.

When a muscle contracts under real load, it secretes signaling proteins into the bloodstream. Some of them cross into the brain and increase the primary growth factor for neurons. In plain terms, when you load a bar and stand up with it, your legs send a chemical message to your head that says *grow* [Wrann et al., Cell Metab. 2013; Moon et al., Cell Metab. 2016].

Skip the lift and the message does not get sent. That is the whole mechanism, and it is why “staying active” — golf, a Saturday hike, ten thousand steps — is not a substitute. Those are excellent things. They are not load.

The evidence on the outcome side is what it is, and I will state it the way Chapter 9 requires. Pooled across meta-analyses, people with higher physical activity carry a relative risk for all-cause dementia of about 0.80 — roughly a fifth lower — with leisure-time activity specifically closer to 0.76 [Iso-Markku et al., BJSM 2022; Feter et al., Lancet Public Health 2026]. Those are observational estimates with all the confounding that implies, and I told you in Chapter 9 that the honest aggregate across domains is 30%, not more.

The program is five movement patterns — a squat, a hinge, a push, a pull, and a carry — two sessions a week, three to five sets each, five to ten repetitions taken close to but not into failure, with a little more weight or one more repetition every week or two.

It is not bodybuilding. You do not need a chest day. It is not the class your kids take, and it should not be, because one torn rotator cuff at fifty-eight buys twelve months out of the gym, and twelve months out of the gym is twelve months of the loss described below.

Forty-five minutes, twice a week. Six and a half hours a month, every minute of it new.

The quiet loss, which is the real exposure

Here is the reason the strength line is not optional at your age.

Past fifty, without resistance training, the average person loses roughly one percent of their muscle mass a year, accelerating toward two percent in later decades [Wilkinson et al., Ageing Res. Rev. 2018]. Quietly. Painlessly. Invisibly under a sport coat. Run that forward from fifty to seventy and it is a third or more of the lean tissue you are carrying right now.

Muscle is not decoration. It is where glucose goes after a meal, which is most of what stands between you and the metabolic

road in the next chapter. It is the scaffolding that decides whether a fall at seventy-eight is an inconvenience or the beginning of the end. Grip strength alone is one of the cleanest single predictors of all-cause mortality in adults past fifty — a better one, in the large international cohort that established it, than systolic blood pressure [Leong et al., *Lancet* 2015].

And it is cognitive reserve. Two people can carry similar amounts of underlying pathology and function very differently, and the differences travel with things like this.

Which produces the cruelest sentence in this whole subject, and I am going to say it plainly because Chapter 4 already told you the same thing from the other side: the fitter the body, the longer the decade. A physically robust man who develops dementia at seventy lives longer with it, and the ledger runs longer. Strength alone is not the answer to that. Strength inside the full schedule — with the metabolic road in Chapter 12 and the sleep in Chapter 10 — is aimed at the diagnosis arriving later or not at all, which is a different bet than arriving on time with a good bench press.

I would rather hand you that sentence than let you find it yourself in year nine.

The walk, and the thirty grams that follow it

Twenty minutes, five mornings, before anything crosses your lips. Outdoors, for the light, which is doing the work of the last chapter at the same time.

Fasted is not a fad and it is not the gym-rat version of fasted. It is a walk. Overnight, insulin is at its low point, and movement in that state pulls preferentially at stored fat — the compartment we actually care about, for reasons that are in Chapter 12. It costs nothing, requires no equipment, and doubles as your circadian anchor. Of all the items on this schedule it is the one I have seen change the most for the most people with the least argument.

Then, after the walk, you break the fast — and you break it with **thirty to forty grams of lean protein before anything else goes in.**

Not because protein is virtuous. Because there is a threshold. Below a certain dose, the signal that tells muscle tissue to maintain itself fires weakly and the body shrugs. That threshold rises with age: twenty grams was enough at thirty, and thirty to forty is the floor at fifty-five. Below it you are training and then failing to send the message the training was for.

Five ounces of chicken. Four or five ounces of beef. Five ounces of salmon. Five or six eggs. None of it exotic, none of it expensive when it is bought deliberately.

And then a window. Most people do well eating between roughly nine in the morning and six in the evening — about nine hours of feeding, most of the fasting done unconscious. Not because clocks are magic, but because insulin sensitivity is genuinely better earlier in the day, and because eating at nine at night puts digestion in direct competition with the sleep architecture from the last chapter.

Visceral fat, and where medication fits

Now the measurement that matters more than the scale.

The fat around your organs is not the fat under your skin, and they behave completely differently. Abdominal fat is not stored calories sitting inertly in a warehouse. It is endocrine tissue. It secretes inflammatory signaling molecules around the clock, it drives insulin resistance both in the body and in the brain, and in a man it runs an enzyme that converts his own testosterone into estrogen — which is why the loop tightens on itself and why so many men in their fifties cannot get out of it with effort alone.

The scale can hold perfectly steady while all of that gets worse. Measure the waist.

And here is where I have to be honest about something the fitness industry will not tell you. For many people past fifty, diet and exercise alone do not clear this on any reasonable time-

line. Testosterone has been drifting about a percent a year since thirty. The satiety signal is muted. Insulin has been running elevated most of the waking day for two decades. That is not a character problem. It is a physiology problem, and telling a fifty-eight-year-old to try harder against it is the same advice that has already failed him twice.

For some people, a licensed clinician will determine that weight-management medication is appropriate. That class of drug works by amplifying a satiety hormone your own gut already makes — it is not a stimulant and it is not a cheat code, and it belongs inside the rest of this schedule rather than instead of it, because dropping intake sharply without the protein rule and the strength sessions costs you lean tissue you cannot spare. Whether it applies to you is a determination made by an independent prescriber after labs and an actual evaluation. Not by this book, and not by an advertisement.

Budget \$150 to \$300 a month if it is indicated. Budget nothing if it is not. Most households use one prescription category on this whole schedule or none.

What the blank line looks like

Leave this line blank and the ledger writes itself in the middle column of Chapter 4.

Muscle comes off at one to two percent a year, unnoticed, for twenty years. Glucose handling degrades because the place it

was supposed to go is smaller. Visceral fat accumulates and starts producing its own inflammatory load. Then a fall at seventy-eight, or a hip, or the pneumonia after the hip — and the hundred days of skilled nursing that Chapter 2 told you not to count on gets consumed, and the household comes out the other side needing hours instead of appointments.

Twenty hours a week of in-home help runs \$36,400 a year at 2025 national medians, and every additional hour is another \$1,820 a year, forever [CareScout 2025]. That is the phase change. It arrives through the body long before it arrives through the diagnosis.

Why this line never closes

Muscle is the only thing on this schedule you build rather than rent — and it is also the only one that begins spending itself down the moment you stop.

Past fifty you are either adding to that account or drawing on it. There is no flat. The body's own production of the compounds that support the tissue declines with each decade, the anabolic threshold rises, and the tissue turns over whether or not you asked it to.

Ninety minutes a week at fifty-eight is still on your balance sheet at seventy-two. That is the closest thing to compound interest that exists in a body, which I told you in Chapter 7 —

and the corollary is that the compounding runs the other direction with exactly the same patience.

The Premium

The premium: nine hours a month — two forty-five-minute strength sessions a week, a twenty-minute fasted walk five mornings. In dollars: \$30–\$120 a month for a gym, \$160–\$560 a month for coaching if you use it, and the food is already in your budget. Plus \$150–\$300 a month if an independent licensed clinician determines weight-management medication is appropriate for you.

What it buys: lean mass held instead of lost, glucose handled where it is supposed to be handled, visceral fat mobilized, and a relative risk for all-cause dementia around 0.80 in the more physically active — an observational estimate, folded into the conservative 30% of Chapter 9, not stacked on top of it [Iso-Markku et al., *BJSM* 2022]. It also buys most of the dividend in Chapter 6.

What it does not buy: a guarantee, or an exemption from the sentence above. A fitter body that receives the diagnosis anyway lives longer with it. This line is aimed at the odds of the diagnosis, not at the comfort of it.

Now the line that costs almost nothing and that everybody skips.



Chapter 12 — Mitigate: Stop Paying Claims Against Yourself

This line costs essentially no hours, and it is the one nobody funds.

That is not a paradox. It is the entire nature of the line item, because Mitigate is not a list of things to add. It is a list of things to stop. Everything on this page is subtraction, plus a handful of appointments — and subtraction has no calendar entry, no equipment, no gym bag, and nothing to post about.

The framing I use for it everywhere else, and I will use it here: **eliminate the insulting behavior.** Stop hurting yourself first. Then add what works.

You have a vocabulary for this already. You do not reduce your loss ratio by buying more coverage. You reduce it by not filing claims against yourself.

The metabolic road

Nearly every road to a dementia diagnosis runs through somewhere else first, and the somewhere else is a set of numbers you can already read.

The 2024 Lancet Commission's list of fourteen modifiable risk factors is, when you look at it honestly, mostly a cardiometabolic list: hypertension, diabetes, obesity, high LDL, smoking, excess alcohol, physical inactivity [Lancet Commission 2024]. And Chapter 9 gave you the hazard ratios. A type 2 diabetes diagnosis at sixty to sixty-nine carries a hazard ratio of 1.70 for subsequent dementia; at fifty to fifty-nine, 1.72; before fifty, 1.90 — each measured against a diagnosis at seventy or later. Obesity together with diabetes diagnosed before fifty carries a hazard ratio of 3.05 against non-obese diagnosis at fifty or later [Qi et al., PLOS ONE 2024].

Roughly a tripling. From two conditions that are both largely addressed by the nine hours in the last chapter.

So the four gauges on this line are not exotic and your physician already draws three of them:

A1c. The three-month average of your blood sugar. Bill's came back at 5.9 when he was fifty-five and somebody said *border-line*, and he heard *fine*, and nobody ever mentioned it again. Know the number, know the direction it has been moving over three draws, not one.

LDL and the rest of the lipid panel. On the Commission's list as of the 2024 update. This is a conversation with your physician about a target, not a conversation with a book.

Blood pressure. The single most modifiable item on the entire list, and the one most likely to be sitting untreated at 148 over 90 in a man who describes himself as healthy.

Waist. Which the last chapter already covered, and which nobody's chart records.

Four gauges, a baseline column, and a column twelve weeks later. That is the whole instrument panel.

The two insults, honestly

Alcohol. I am fifty-nine. I grew up in the same culture you did, where the after-work pour was a reward and the dinner-party wine was a social contract, and I am not going to sit outside that and lecture you.

But I am going to be specific, because you are entitled to the number. Somewhere between three and seven drinks a week, the evidence is genuinely contested and I will not pretend otherwise. Above seven, and especially in the ten-to-fourteen zone where a lot of successful households quietly live, the picture sharpens: measurable effects on the memory structure of the brain on imaging, on sleep architecture, on hormones, and a cancer risk that climbs in roughly linear fashion. The large

2018 analysis that ended the red-wine story concluded that the level associated with the lowest overall harm is zero [GBD Alcohol Collaborators, *Lancet* 2018].

The useful move is not abstinence. It is dropping a tier. Cutting the real number in half, sustained, moves sleep, blood pressure, hormones, and the gut barrier at the same time — four line items with one decision.

And find out what the real number is, because it is almost never the number in your head. Thirty days on the calendar, then a month of actually writing it down, will tell you more than any lab draw on this page.

Smoking and nicotine. On the Commission's list, in every version, and there is nothing to add. If this applies to you it is the largest single item on your schedule and it dwarfs everything else in this book.

Hormones and the two canaries

Now the part of this line that most households will not raise on their own, and where the physiology is genuinely sex-specific. I am going to write it that way rather than write something vague enough to cover both.

For the man reading this

There is a signal your body files years before anything shows up on a memory test, and it is almost never discussed as a

signal.

The arteries that produce an erection are roughly one to two millimeters across. The coronary arteries are three to four. The same process that ends in a heart attack — the softening of the single layer of cells lining every vessel you own — narrows the smallest vessels first, because they have the least margin. Which is why erectile difficulty frequently precedes overt cardiovascular symptoms — commonly by two to three years, sometimes longer [Montorsi et al., *Eur. Urol.* 2003; Thompson et al., *JAMA* 2005].

That is not an embarrassment. That is a stress test your own body ran and reported the results of, three to five years early, for free. Almost no other system in the body gives that kind of notice.

And the brain is on the same chassis. The brain has more vascular surface area per gram than any organ you own. The same endothelial function, the same inflammatory load, the same insulin resistance. Men who tell me their erections have softened over two years almost always tell me, in the next breath, about the word they could not find in a meeting. They believe those are two problems. They are one fire in two rooms.

The response is not a pill that silences the alarm and leaves the fire burning. It is a panel — total and free testosterone, estradiol, SHBG, thyroid, morning cortisol, DHEA-S, alongside fasting glucose, A1c, lipids, and an inflammatory marker — read together, by someone actually looking, against your symptoms.

Where hormone support is indicated, it is prescribed by an independent licensed clinician after that evaluation, monitored, and written to you. Where it is not indicated, it is not prescribed, and any operation that hands out the same protocol to everyone who fills out a form is not doing medicine.

For the woman reading this

Your body files a different report, and it is quieter, which is why it gets misread for years.

It arrives as a cluster over about eighteen months, and no single piece of it looks like a medical event. Sleep breaks first — asleep fine, awake at two or three, ninety minutes of lying there. Then the cognitive symptoms, which frighten women more than anything else on the list: word-finding lapses, a name gone for four seconds, walking into a room and losing the reason. Then a shorter fuse and a flatness where enthusiasm used to be. Then the same calories and the same walking, and the weight relocates to the middle. Then intimacy changes and nothing gets said, because there is no script for raising it in an eleven-minute appointment.

Estrogen is not a reproductive hormone that happens to have effects elsewhere. Its receptors sit densely in the memory structure and the executive structure of the brain, it supports blood flow through the same vascular pathway I just described in men, and it helps keep the brain's own immune cells in a calm state rather than an activated one. When it falls — not gradually across decades, but over a compressed handful of

years — the brain goes through a real metabolic transition. The fog is not imagined.

And roughly two-thirds of Alzheimer's cases in this country are in women [Alz. Assoc. 2026]. Longer life expectancy explains part of that gap and, on the honest reading of the literature, not all of it. The menopausal transition is the leading suspect, and the argument is not settled.

Timing appears to matter with hormone therapy — starting near the transition, in a woman whose vascular system is still healthy, looks like a different proposition than starting a decade later. That is a hypothesis, not a settled fact, and nobody honest will tell you hormone therapy prevents dementia. But the window is real enough that *waiting to see how it goes* is itself a decision with consequences, and it should be made deliberately with a clinician rather than by default. It is also individualized, and there are real contraindications — a personal cancer history, a clotting history, certain cardiovascular and liver conditions — which are a clinician's call after reviewing your history, not a book's.

Two birds, two songs, the same gas in the same shaft.

And for the household already past the warning

There is one more category, and I am going to describe it generically on purpose. For people already carrying a neurological diagnosis, there are regenerative therapies — a field

that is moving fast enough that naming a modality in a printed book would be irresponsible. It is a real conversation to have with a licensed clinician, it is expensive, and it is a different conversation from this schedule. Everything current is at my4mlife.com.

Air and water, since you asked what else is free

The environmental piece belongs on this line because it is the same shape: it is subtraction, and it does not hurt, which is why nobody does it.

Indoor air was measured as two to five times more polluted than outdoor air in the EPA's 1989 Report to Congress, and separate research puts the average American's time indoors at roughly eighty-seven percent, commonly rounded to ninety [EPA 1989; Klepeis et al., JEAEE 2001]. Fine particulate matter is on the Lancet Commission's list as air pollution, one of the fourteen [Lancet Commission 2024]. The floor is a HEPA-plus-carbon filter in the bedroom, because that is where you spend a third of your life breathing the same air for eight hours. A few hundred dollars, once.

Water is the same logic. Municipal treatment in this country was designed to stop cholera, and it does that superbly. It was not designed to remove the categories of compounds that show up in tap water now. A countertop reverse-osmosis unit is a

few hundred dollars and takes care of the categories that matter.

The phone out of the bedroom is free. Ten minutes of bare feet on grass during the morning walk is free.

I put these last, and I keep them short, because I do not want a man who reads statements to conclude this book is about water filters. It is not. But the line is cheap, the mechanism is plausible, and cheap bets with plausible mechanisms on a risk with no carrier behind it are exactly the bets your whole career says you should be taking.

What the blank line looks like

Leave this line blank and here is the ledger entry.

An A1c that goes 5.7, 5.9, 6.2, 6.5 across eight years with nobody watching the slope. A blood pressure sitting untreated. Two drinks a night for twenty years. A canary that sings at fifty-four and gets answered with a prescription that silences it. A hormonal transition that gets six years of *manage your stress*. And a fourteen-item list of modifiable risk factors, of which this household addressed none, on a risk with a 42% lifetime probability after fifty-five and roughly 66% for a couple [Fang et al., *Nat. Med.* 2025].

Nothing on that list produced an event. All of it produced a slope.

Why this line never closes

Because the insults never stop arriving.

The hormonal curve runs one direction after thirty and does not turn around on its own. The environmental load is cumulative — the dose is the duration. The gut takes a hit every week of high stress and every course of antibiotics for the rest of your life. And the drink that comes back into the rotation in a hard quarter comes back at the same cost it always had.

This is the line item you re-audit every year, the way you re-audit everything else in that folder. Which is the point of putting it on a schedule instead of in a resolution.

The Premium

The premium: almost no hours. Four gauges measured, three appointments made, one drink tier dropped, one panel drawn. In dollars: the labs and consults, a few hundred once for a bedroom air filter and a water unit, and \$150–\$350 a month if an independent licensed clinician determines hormone support is appropriate for you.

What it buys: the largest share of the fourteen modifiable risk factors the Lancet Commission identified, addressed at the individual level [Lancet Commission 2024]; a metabolic road with hazard ratios of 1.70 to 3.05 attached to it, not taken [Qi et al., PLOS ONE 2024]; and a warning signal read as a warning rather than silenced.

What it does not buy: the 45% headline. That number is a population ceiling under conditions that will never obtain, as Chapter 9 showed you with the arithmetic. Thirty percent stays the working assumption, and none of this prevents, treats, cures, or reverses anything. It lowers the odds.

Which leaves one line on the schedule. It is the shortest one, it costs nothing, and without it none of the other three get paid.

Chapter 13 — Motivate: The Face on the Other Side of the Table

Everything in this book so far has been an argument.

I have priced an exposure, read a coverage schedule, told you what happened to the market, walked a decade in installments, shown you the second patient, run the expected-value math in front of you and admitted where it comes up short, and laid out three line items with hours and dollars attached.

If arguments worked, that would be enough, and this chapter would not exist.

It is not enough. It has never been enough, for anyone, and I have watched enough capable people get persuaded in a room and then not do the thing to have stopped believing otherwise. It is never that they stopped believing the numbers. It is that at six-fifteen on a Tuesday morning in March, a thing you are doing to avoid a statistic in 2044 loses to a warm bed. Every time.

It is not a character defect. It is the correct behavior of a nervous system that discounts the future, and it will beat your ledger in a straight fight for the rest of your life.

So the fourth line item is the engineering that keeps the other three funded.

Compliance is built, not summoned

Start by dismantling the thing you are probably carrying: that the people who run this for decades have more discipline than you do.

They do not. They burned through their discipline years ago, same as everyone. What they built instead is a system in which doing the thing is the path of least resistance. The protein is in the refrigerator because somebody put it there on Sunday. The two sessions are in the calendar, with names, like meetings. The supplements are in a case. Somebody knows when they missed.

Motivation, as a feeling, is weather. Compliance, as a structure, is bedrock. You have never once run your business on how you felt about it on a given Tuesday, and you should not run this on that either.

Three load-bearing components. A named person. A sentence about who you are. And fifteen minutes on a Sunday.

The named person

Here is the question, and it is not the one you expect.

Not *why are you doing this*. **Who are you doing this for?**

Name one. Your wife. Your daughter. Your son. Your mother, who you are already watching. Your best friend, who died of something that had a name and a modifiable risk factor list. Or yourself, at seventy-eight, in specific detail — walking your own grandchildren somewhere, still the one people call when something has to be decided.

One person. One face. Someone you can picture in a room.

A “why” with nobody on the other end of it does not survive February. People write *I want to be healthy* and three months later cannot remember writing it, because there is no one waiting for an answer. The behavioral literature on this is not subtle: external accountability beats internal commitment every time the contest is run. Human beings did not evolve to keep promises to themselves. We evolved to keep promises to each other.

And there is a second reason for the question, which is specific to this book and which you have already read.

Chapter 5 told you who actually pays for the uninsured decade. It is not you. It is the person who did not get diagnosed — a 63% higher four-year mortality among strained spousal caregivers, in the landmark 1999 study that remains the best evi-

dence we have [Schulz & Beach, *JAMA* 1999]; chronic conditions at nearly twice the rate of non-caregivers; a decade of ranking herself last inside a set of decisions where every individual call was correct [Family Caregiver Alliance].

You have spent thirty years buying down risk on her behalf. The term life was never for you. You were never going to see a dollar of it.

So when I ask who you are doing this for, I am not running a motivational exercise on you. I am asking you to notice that you already know the answer, and that it is sitting one chair over.

There is also, unhelpfully for my purposes, real evidence that having an answer matters on its own. In a long-running cohort study of older adults, people scoring high on a validated measure of purpose in life were roughly two and a half times less likely to develop Alzheimer's over the follow-up window than those scoring low, after controlling for age, education, depression, and baseline cognition — and later work from the same cohort found high-purpose individuals showed less clinical expression of the same underlying pathology [Boyle et al., *Arch. Gen. Psychiatry* 2010]. Two people with similar findings in the tissue, functioning differently in life.

I do not file that under inspiration. I file it under physiology. It behaves epidemiologically like a modifiable risk factor, which puts it on the same schedule as everything else in Part III — and it is the only line item that costs nothing at all.

The sentence

Once the face is named, write one sentence about who you are. Present tense. Not a goal.

The distinction is not semantic. *I am trying to lift twice a week* is a behavior, and behaviors negotiate with you on bad days — you were only trying, and today was not the day. *I am someone who lifts* is an identity, and skipping is then dissonant, and your brain hates dissonance enough to route you back. When behavior and identity conflict, identity wins almost every time.

So: *I am the one who protects his mind. I am someone whose mornings start with protein. I am not the man who has two drinks a night anymore.*

Write it down. Read it on Sunday. Read it on the third Tuesday in February. Read it the morning after a wedding where you want to throw the whole project out.

And do not build the sentence out of a diagnosis. *Pre-diabetic* is not an identity. *Low T* is not an identity. Those are readings on a gauge. The sentence has to have motion in it — *I used to be soft, and I am the one rebuilding* — because a sentence with motion survives a bad quarter and a label does not.

That is where the line on the cover of my other book actually lands, and it is not a slogan. **Don't lose your identity and your dignity while you still have a choice.** The ledger is

about the money. This is about the choice, and the choice is exercised at the level of the sentence you use to describe yourself.

Fifteen minutes on a Sunday

The last component is the cheapest thing in this entire book and it is the one that determines whether the other nine hours actually happen.

One sitting. Usually Sunday. What is on the calendar this week, what is getting cooked, where the two strength sessions actually go — written into the same calendar that runs everything else in your life that gets done. Not a separate app. Not a notebook. The calendar.

Fifteen minutes a week. One hour a month. Zero dollars.

Then tell one person. The named one, preferably. The “why” that lives only in your own notebook is half a “why.” The one somebody else has read is the one that gets you out of bed on the third Tuesday in February.

The conversation Bill and Cindy never had

I want to close Part III with the one scene I have been holding back.

Bill was fifty-five once. Thirty pounds heavier than he had been at forty, an A1c of 5.9 that somebody called borderline, sleeping badly and calling it a personality trait, three years off the tennis court because of a knee he never had looked at.

Cindy noticed all of it. She noticed it the way spouses notice — early, accurately, and without a vocabulary that would not sound like nagging. Twice that year she got most of the way to saying something. Once in the car coming home from a wedding, and once in the kitchen on a Sunday, and both times she chose the other thing, because he had a Thursday and a business and a bad shoulder and a hundred reasons to hear it as criticism.

That is the conversation. It is not dramatic. It never is. It is a woman deciding, twice, that this was not the moment — and then a decade going by in which the moment never got better, and then a neurologist's office in a building with a parking garage, and a word.

Sixteen years later she is seventy-nine, half a million dollars of the retirement they built together is gone, and the thing she thinks about is not the money. Chapter 5 already told you what it is. It is the year she was grieving a man who was still sitting at the table.

I cannot tell you that the conversation would have changed the diagnosis. Nobody can tell you that, and Chapter 9 is where I showed you exactly how much and how little the evidence supports. What I can tell you without any statistical hedging at all

is that the thirteen years between fifty-five and sixty-eight would have been materially better years — for him, and for her — and that those years were never contingent on anything.

If somebody handed you this book and said *read the first six pages*, that was the conversation. It is happening right now. You are holding it.

So finish the line item. Put down the book, pick the face, and say the name out loud.

I am doing this for: ____.

The Premium

The premium: one hour a month and no dollars at all. One named person, one present-tense sentence, fifteen minutes on a Sunday, and one other human being who knows the plan exists.

What it buys: the other three line items, actually paid, in year four — which is the year everything in Part III stops being a plan and starts being a result. And a construct that behaves in the research like a modifiable risk factor in its own right, at a hazard that is not small [Boyle et al., Arch. Gen. Psychiatry 2010].

What it does not buy: a substitute for the work. A named face does not lift the weight, and no sentence about who you are prevents, treats, cures, or reverses anything. It makes the premium payable for twenty years instead of eleven weeks. That is all it does, and without it nothing else on the schedule gets funded.

That is the schedule of benefits. Four lines, ten hours, and a premium denominated in something no carrier can reprice and no underwriter can decline.

All that is left is signing it.

Part IV — Signing

Chapter 14 — Begin with the End in Mind

There is a paperback somewhere in your house with a tan cover and a cracked spine.

You read it in 1989, or 1990, or somewhere in there — on a plane, probably, or at a sales conference where somebody handed out copies. Habit 2. *Begin with the end in mind*. Covey's exercise was to sit at the back of the room at your own funeral, listen to four people speak, and ask yourself what you would want them to be able to say. Then work backward from that.

You did not just read it. You used it.

You built the business plan backward from the exit. You wrote the buy-sell agreement backward from the day one of you was gone. You have an estate plan, a trust, a durable power of attorney, and a letter in a fireproof box that tells your daughter where everything is. You know the number you needed at sixty-five and the year you hit it. You have, if you are like most of the men I have had this conversation with, already decided where you are going to be buried.

That is thirty years of beginning with the end in mind, executed with more discipline than most people bring to anything.

And there is exactly one thing you never ran the exercise on.

The instrument that was supposed to be doing the living.

You planned the estate. You did not plan the mind that was going to spend it. Every document in that fireproof box quietly assumes a version of you at seventy-eight who can still read the document, still sign it, still be the one in the room whose opinion settles the question. Nobody stress-tested that assumption. It was not on the checklist, because no advisor has a product to sell against it, and because there is no line on any balance sheet for it.

That assumption is what this entire book has been auditing.

The end, actually stated

So state it. Not the funeral — that is the wrong end for this particular exercise. The funeral is one afternoon and you will not be at it. The end that decides everything is the decade before it.

Write yours down. Here is a draft you can steal and edit, which is how most people get started:

The last ten years, sharp. On my feet. In my own house. Making my own decisions, and still the one people call when

something has to be decided. And my wife's last decade spent as my wife, not as my nurse.

Read that back and notice what is not in it. There is no number on a scale. There is no age. There is nothing in it about living longer — you can add years to the end of that sentence and make it worse rather than better, which is the whole argument of Chapter 6.

Then notice the thing that took me an embarrassingly long time to see myself.

The destination is a mind.

Not the money the mind manages. Not the body that carries it around. Not the house, which is only in the sentence because being in your own house at eighty-four requires that you can still be trusted to run a stove. Every item in that paragraph is downstream of one organ, and it is the organ nobody built a policy for.

That is why the schedule in Part III is shaped the way it is. Muscle, Mitigate, and Motivate are the three line items. Mind is not a fourth item sitting alongside them — it is what the other three purchase. *Begin with the end in Mind*. Both readings at once: start from the destination, and understand that the destination is the mind itself.

You have been running Covey's habit on everything you own for thirty years. This is the one asset you left off the list.

The circle

One more way to hold it, and then I will stop being literary at you.

A long crossing is not measured in distance. It is measured by whether the man who comes back into the harbor is the same man who left it. That is the only test that has ever mattered on a voyage, and everything else — the cargo, the weather, the log — is bookkeeping.

You set out from that harbor at twenty-two with nothing. You are somewhere out past the middle of it now. The circle closes when you come back in under your own power, recognized, recognizing.

That is the end in mind. The rest of this book has been about provisioning the ship.

The face

Chapter 13 gave you the machinery. This is where you actually run it.

Not *why are you doing this*. **Who are you doing this for?**

One person. Not a category — not “my family,” which is four people and therefore nobody. One face you can put in a room. Your wife at the kitchen table. Your daughter, who is going to be the one who drives you. Your son, who will inherit the pat-

tern along with the house. Or yourself at seventy-eight, in enough detail that you could describe what you are wearing.

Write the name on the line. Physically. In the book, in pen.

I am doing this for: ____.

Then the half that people skip, which is the half that works: tell them. Out loud, to their face, this week. Not a speech. One sentence — *I read something, I'm going to start doing a couple of things, and I wanted you to know so somebody else knows.*

A commitment that lives only in your own head is not a commitment. It is a preference, and preferences lose to February.

The sentence

Then one line about who you are. Present tense, with motion in it, never built out of a lab value.

I am the one who protects his mind.

I am someone whose mornings start with a walk and thirty grams of protein.

I used to be soft, and I am the one rebuilding.

Write it where you will see it. Read it on a Sunday. Read it the morning after the wedding when you want to throw the whole project out.

Those are the two things you sign before day one. A name and a sentence. Neither one costs a dollar and neither one takes ten minutes, and in four years they will turn out to have been the load-bearing part.

Bill, at fifty-five

I have one thing left to say about Bill, and then he can go.

Two readers are holding this book right now, and they need different things from this page.

If you are Bill at seventy-one — if the word has already been said in an office in a building with a parking garage, in your household or your mother's — then the arithmetic in Part I is not a forecast for you. It is an invoice, and I am not going to pretend otherwise on the last pages of a book. But read Chapter 5 again, because the second patient in your house has not been diagnosed with anything, and every line on the schedule in Part III still applies to her at full strength, and she is currently the only person in the marriage not being cared for. And when you are ready to ask what medical support is available for a household already past the diagnosis, ask a licensed clinician, in an actual evaluation — there is more available now than there was five years ago, and none of it is a cure, and I am not going to sell you one on page two hundred.

But most of you are Bill at fifty-five.

Thirty pounds up. An A1c somebody called borderline, which you heard as fine. Sleeping badly and calling it a personality trait. A knee you never had looked at, three years off the court, and a Thursday. There is nothing wrong with you that anyone has put a name to. That is precisely the problem: nothing has a name yet, which is the only window in which any of this works.

And here is the part I want you to sit with.

Cindy tried twice.

Once in the car coming home from a wedding, and once in the kitchen on a Sunday, and both times she chose the other thing, because you had a business and a bad shoulder and a hundred reasons to hear it as criticism. She was not being timid. She was being married to you, which is a job that involves a great deal of judgment about which Tuesday is the right Tuesday.

She never got a third try in that story. In yours, she just did — because somebody handed you this book, or left it on the counter, or mailed it to you without a note.

So finish the conversation, since you are the one holding it.

You do not have to promise her anything. Do not promise her anything; you have made promises about your health before and you both remember how they went. Say the smaller, truer thing instead:

I read it. I'm going to take the seven-minute assessment tonight, and I'll show you what it says.

That is the whole conversation. It is not dramatic. It never is.

Then turn the page, because the next chapter is thirty days long and it starts tonight.

Chapter 15 — The First Thirty Days

Coverage does not begin when you decide to buy it. It begins when the first premium clears.

So this chapter is a schedule. Thirty days, four weeks, in the order the parts actually stack, with nothing on it that requires you to become a different person first. It is deliberately smaller than you are expecting. That is not modesty — it is the reason it works. Every one of these is binary: it happened or it did not, and on the thirtieth day you can audit yourself in four minutes.

Write real dates in the margin. A plan without dates on it is a mood.

Week 1 — Baseline

You cannot manage what you have not measured, and you have never measured this one. That is the whole job of week one.

Day 1. Take the assessment. Go to my4mlife.com/assessment. Twenty questions, about seven minutes, on your phone, tonight. It gives you a score out of 100, the band you are in, and — the part that actually matters — your top three. Not twenty things to fix. Three, ranked, in the order that will do the most for you.

It is free, and it is also the door: taking it sets up your account, so the app, the Logbook, a copy of *Begin with the End in Mind*, and the weekly Zooms come with it. There is nothing to buy on that page. If you were braced for a checkout at the end, there isn't one.

Do it before you do anything else, because everything below is easier when you know which three lines are yours.

Day 1. Set the two anchors. They are the cheapest things in the book and they are both about the clock.

Light. Ten minutes of outdoor light within an hour of waking. Not through a window. This one anchors the body clock that runs everything downstream of it, and it rides along free with the walk below.

Sleep. One fixed wake time, seven days a week, including Saturday. Not a bedtime — a wake time. The bedtime follows it on its own within about ten days, and fighting the bedtime directly never works.

Day 2. Put the sessions in the calendar. Two forty-five-minute strength sessions a week, in the same calendar that runs the rest of your life, with names, like meetings. Not “workout.” *Monday 6:15 — Strength.* You do not miss meetings. You miss intentions.

Day 3. The walk begins. Twenty minutes, outside, before you eat. Five mornings a week from here on out. It is not a workout and you are not trying to raise your heart rate — you are walking, fasted, in daylight, which is doing three different jobs at once for zero dollars.

The first week is the hardest one, and it is hardest for a stupid reason: it is boring, and nothing has happened yet.

Day 7. Tell the named person. The name you wrote in Chapter 14. One sentence, out loud, to their face.

By the end of week one you have a number, three priorities, two calendar entries, and one other human being who knows the plan exists. You have not changed your diet. Leave it alone for now.

Week 2 — The kitchen

Now the food, and only one rule at first.

Protein first. You break the fast after the walk, and you break it with thirty to forty grams of lean protein before anything else goes in. Not coffee and toast. Not cereal. This one rule reorganizes the rest of the day's eating without your having to think about the rest of the day's eating, which is why it is the only one I am giving you in week two.

The grocery list changes before the meals do. This is the practical trick and almost nobody is told it. You do not start by cooking differently; you start by shopping differently, once, and then the kitchen makes the decision for you all week. Protein at the front of the cart in quantity. Vegetables. Something for the first thirty grams that requires no cooking at six-fifteen in the morning. And the two or three items you already know are in there for no reason other than habit — you know exactly which ones — do not make the trip this time.

One insult, removed. Pick one from Chapter 12 and take it off the board this week. The second drink. The eleven-o'clock screen. The three-o'clock pastry that is why four o'clock happens to you. One. Not four. You are not trying to be good; you are trying to still be doing this in March.

Day 14. Fifteen minutes on a Sunday. The planning hour, which is not an hour. What is on the calendar, what is getting cooked, where the two sessions actually sit. This is the appointment that keeps all the others.

Week 3 — Load, and the labs

***The sessions get heavy.** Two a week, forty-five minutes, compound movements — push, pull, hinge, squat, carry — loaded so the last two repetitions are genuinely difficult. That difficulty is the active ingredient. Two sessions run seriously beat five run conversationally, and it is not close. If you have not trained in years, spend this week learning the movements at a weight that feels insulting. The loading comes in week four and it will still be there in year ten.*

Get the panel drawn, if it is indicated. A baseline is a lab draw and an actual evaluation by an independent licensed clinician — the metabolic markers, the inflammatory markers, and the hormone panel, read against what is optimal for you rather than against a reference range built from a population that is not aging well. What that panel says, and whether anything on it warrants prescription support, is a determination made by a licensed prescriber after seeing you. It is not made by this book, and it is not made by an advertisement. If you do not have a clinician for this, start at my4mlife.com/consult.

Book the three appointments. Hearing test. Eye exam. A periodontal probing — not a cleaning, a probing. Three phone calls, most of it covered, and three of the fourteen risk factors

from Chapter 12 closed. Nobody in your age bracket does all three. Be the one who does.

Week 4 — The household ledger

The last week is the one that makes this a household decision instead of a private project, and it is the week most men skip.

Do not skip it.

Run your own number. Go to my4mlife.com/go/uninsured-decade and put in your actual inputs — your age, your spouse's, your state, roughly where you sit financially. The ten-year ledger in Chapter 4 is a national median household. This one is yours, on your state's care costs, with the risk-reduction assumption set where you want it set and visible on the screen while you move it.

Print it. There is a button on the page for exactly this. One sheet.

Then have the meeting. Sit down with your spouse or partner, and then with whoever handles your money, and put that sheet on the table next to the coverage inventory you filled out in Chapter 1. Ask the three questions in the order they actually go: *What does our plan assume about this? What happens to the survivor if it lands? What are we doing about the only lever we have?*

Your advisor may not have an answer for the first two. That is not a failure on their part — there is no product for it, which

has been the point since page one. But the conversation gets held, in front of the person who would otherwise have to hold it alone in twelve years, and the sheet goes in the folder with the policies where it belongs.

Show up to one Zoom. Camera on or camera off. You will be in a room with other people running the same schedule, which does more for week six than anything I can write here.

Day 30: what you should actually feel

I am going to be careful here, because the temptation at the end of a book is to promise you a transformation, and thirty days does not produce one.

Here is what will not have happened. You will not have lost thirty pounds. Your A1c has not moved much, because that number reflects about three months and you have given it one. You will not look different in a photograph, and nobody at work will comment.

Here is what will have happened, in most people, and it is the reason I put it last.

You will be sleeping. Not perfectly. But consolidated — you go down, and the next thing is the alarm, instead of the ceiling at one-forty and again at four. Some people get it in ten days and some in five weeks. It is almost always the first thing to

move, and it is almost always the thing that convinces a skeptical man that any of the rest of this is real, because it is not a number somebody handed him. It is his own Tuesday, arriving different.

Then the four-o'clock comes back. Then, somewhere around the eighth or tenth week, you carry something up a flight of stairs and notice you did not think about it first.

That is the dividend, arriving early and on its own schedule, on a premium you have been paying for one month.

The other payout — the contingent one, the ledger that never gets written — you will never be able to prove you collected. That is the nature of every policy in that folder. You will only be able to notice, at eighty-two, that the invoice never came.

The 30-Day Ledger

	Item	Date done
☐	Assessment taken — score, band, top three	_____
☐	Wake time fixed, seven days	_____
☐	Ten minutes of morning light	_____
☐	Two strength sessions on the calendar, by name	_____
☐	First fasted morning walk	_____
☐	Named person told, out loud	_____
☐	Identity sentence written down	_____
☐	Protein first — thirty to forty grams after the walk	_____
☐	Grocery list rewritten once	_____
☐	One insult removed	_____
☐	Fifteen minutes on a Sunday	_____
☐	Two sessions run, loaded	_____
☐	Labs drawn / evaluation booked	_____

	Item	Date done
☐	Hearing, eyes, periodontal probing booked	_____
☐	Own ten-year number run and printed	_____
☐	Household ledger conversation held	_____
☐	One Zoom attended	_____

Seventeen lines. Nine of them are free. None of them take more than an hour.

That is the whole first premium payment, and it is the only one you have to make right now.

The Premium

The premium: roughly ten hours a month and roughly eight to ten thousand dollars a year, most of which is money already in your budget under different headings. Two strength sessions, five walks, a rewritten grocery list, three appointments, one Sunday planning block, one named person, one sentence.

What it buys: two payouts. A contingent one — lower odds on the decade in Chapter 4, at the conservative assumption I defended in front of you in Chapter 9, stated as odds and never as a promise. And a non-contingent one that starts inside six weeks and does not depend on the first one arriving at all: sleep, strength, the four-o'clock, presence, an active retirement decade, and a spouse whose last decade is spent as your spouse.

What it does not buy: immunity. Nothing in this book prevents, treats, cures, or reverses dementia, and no one who tells you otherwise is doing math. It lowers the odds. That is the only sentence I have sold you, it is the same one I started with, and it is the last one.

Signing

Every policy you have ever bought was signed at a table by a version of you who did not know whether he would need it.

That is what a signature is for. It is a decision made in advance, by the person who is thinking clearly, on behalf of the person who will not be.

You did it at thirty-four with the term life, when the kids were small and the premium hurt. You did it at forty-one with the disability, and at fifty-two with the buy-sell, and every single time you were signing on behalf of somebody who was not in the room — a widow who did not exist, a partner who might be, a household in a year you could not picture.

This one is the same act. The only difference is that the person you are signing on behalf of is you, at seventy-eight, and there is no carrier on the other side of the table to countersign it.

There is no policy number. There is no folder to put it in. There is a name, a sentence, and a Tuesday morning that is either going to happen this week or not.

I told you on page five that I would not tell you at the end that this pays for itself, and I have not. I told you I would show you the arithmetic with nothing hidden, and I did, including the part where it comes up short. What I will tell you is what I actually believe after thirty years of watching households arrive at this decade either provisioned or not: this is the one exposure on your schedule where the premium is denominated in something no underwriter can decline you for, no carrier can reprice, and no market can leave.

You are the only one still writing this policy.

So sign it. Tonight, in seven minutes, at my4mlife.com/assessment — and then get up tomorrow and go for a walk before you eat.

Don't lose your identity and your dignity while you still have a choice.

— **Dr. TJ**

The Assessment

Everything in this book routes to one page.
my4mlife.com/assessment

Twenty questions. About seven minutes. Free, and there is nothing to buy at the end of it.

What it gives you back, on screen, immediately:

- A **MindSpan Score out of 100** — your baseline, which is the number you do not currently have.
- The **band** you are in, in plain language.
- Your **top three priorities**, ranked. Not a list of twenty things. The three that will do the most for you, in order.

What happens next. Your full results arrive by email, along with the things that come with being a Protégé, at no charge:

- The **app**, which runs the four-month program week by week.
- **The Logbook** — the workbook the program is built around.
- A copy of ***Begin with the End in Mind***, the full-length book behind the framework in Part III.

- **Weekly Zooms**, live, with other people running the same schedule.

If a licensed clinician needs to be involved — labs, hormone support, anything requiring a prescriber — that is a separate conversation at my4mlife.com/consult, with an independent licensed medical practice, after an actual evaluation.

And if you want your own household's ten-year number instead of the national median in Chapter 4, run it at my4mlife.com/go/uninsured-decade and print the sheet for your advisor.

Take the assessment tonight. The rest of it can wait until the weekend. That one cannot, because it is the only line on the schedule that has to happen first.

About the Author

Dr. TJ has spent more than thirty years in clinical practice and now works as a health-span educator, writing and speaking about cognitive longevity for the people who make their household's decisions. He lives and works in Fort Worth, Texas.

He is the founder of **My4MLife**, and the architect of the 4M framework — Mind, Muscle, Mitigate, Motivate — which an assembled team of clinicians, coaches, and researchers built into a working program under his guidance. He does not practice medicine; all medical care, evaluation, and prescribing referenced in his work is provided by independent licensed medical practices.

He is the author of ***Begin with the End in Mind*** and ***Count Yourself Skinny***.

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Sources (stub — full citation list to be completed at manuscript lock)

Every figure above is keyed to one of the following. Full citations, access dates, and the working updated version are maintained at my4mlife.com/science.

- **[Alz. Assoc. 2026]** — Alzheimer’s Association, 2026 *Alzheimer’s Disease Facts and Figures*. Lifetime cost of care \$405,262 (2024 dollars), 70% family-borne; 19+ billion unpaid hours in 2025 valued at \$446.3B. *Its own Alzheimer’s-only lifetime-risk figure (~1 in 5 women, ~1 in 10 men) is superseded in this book by the larger, all-cause-dementia [Fang et al., Nat. Med. 2025] figure below — use that one, not this one, for any lifetime-risk statement.* alz.org/alzheimers-dementia/facts-figures
- **[Fang et al., Nat. Med. 2025]** — Fang M, Coresh J, et al., “Lifetime risk and projected burden of dementia,” *Nature Medicine*, published 13 Jan 2025. ARIC cohort, >15,000 people followed dementia-free from age 55. Lifetime risk of dementia after 55: 42% overall (95% CI 41–43%), 35% men, 48% women; higher in APOE ε4 carriers and Black adults (~45–60%); projected new U.S. cases/yr rise from 514,000 (2020) to ~1 million (2060). For a couple, $P(\text{at least one}) = 1 - (0.65 \times 0.52) \approx 66\%$. **Caveat: all-cause dementia including mild late-life cases — broader than “Alzheimer’s requiring full-time memory care.” The \$405,262 lifetime-cost figure applies to those who go on to develop it, not fur-**

ther-discounted for severity.

[nature.com/articles/s41591-024-03340-9](https://www.nature.com/articles/s41591-024-03340-9); PubMed
39806070; [nih.gov/news-events/nih-research-
matters/risk-future-burden-dementia-united-states](https://www.nih.gov/news-events/nih-research-matters/risk-future-burden-dementia-united-states)

- **[USC Schaeffer 2026]** — USC Schaeffer Center, “Dementia Will Cost the U.S. \$818 Billion in 2026,” June 2026. \$818B total; ~\$143,509 per patient. *Different cost model from the Alzheimer’s Association lifetime figure — not additive.*
- **[Alz. Impact Movement, 2021 data]** — Alzheimer’s Impact Movement, caregiver out-of-pocket spending, \$12,388/yr. **Underlying data year: 2021.**
alzimpact.org/creditforcaring
- **[CareScout 2025]** — CareScout (Genworth) 2025 *Cost of Care Survey*. In-home care \$35/hr; assisted living \$6,200/mo; nursing home private room \$129,575/yr (\$355/day). investor.genworth.com
- **[SeniorLiving.org 2026]** — SeniorLiving.org, Memory Care Costs, June 2026. National median \$8,019/mo; Texas \$6,684/mo.
- **[Medicare.gov, current]** — Medicare.gov, “Long-term care.” “Medicare doesn’t cover custodial care if it’s the only care you need.” Statutory basis 42 CFR §411.400.
- **[TX Medicaid, 2026 figures]** — Texas Medicaid 2026 long-term-care eligibility: \$2,000 individual / \$3,000 couple countable resources; 60-month look-back; CSRA

\$32,532–\$162,660; up to \$4,066.50/mo protected income.
State-specific and illustrative.

- **[Parkinson’s Foundation, 2024 data]** — Parkinson’s Foundation / Michael J. Fox Foundation, *Economic Burden of Parkinson’s Disease*, released 2026 covering cost year 2024. \$82.2B total; \$8.3B care-partner lost earnings; 34% canceled own visits; >20% retired or cut hours.
- **[Schulz & Beach, JAMA 1999]** — Schulz R, Beach S, “Caregiving as a Risk Factor for Mortality: The Caregiver Health Effects Study,” *JAMA*, 1999. 63% increased 4-year mortality among strained spousal caregivers.
- **[Family Caregiver Alliance]** — Family Caregiver Alliance, *Caregiver Health*. Chronic conditions 45% vs. 24% (undated); ~40% of dementia caregivers with depressive symptoms vs. 5–17% of non-caregivers. caregiver.org
- **[Am. Academy of Actuaries]** — American Academy of Actuaries, “Writing Long-Term Care in a Short-Term World.” 100+ carriers circa 2000 to fewer than a dozen writing new standalone policies.
- **[S&P Global; state filings, 2022–2026] / [longterm-caredesk.com, 2026]** — Genworth and John Hancock long-term-care rate filings and approvals.
- **[industry pricing data, July 2026]** — Long-term-care premium ranges for a healthy 60-year-old couple, July 2026 pricing. *Re-verify at manuscript lock.*
- **[CRR / HCG Secure, 2026]** — Center for Retirement Research at Boston College; HCG Secure / Arctos

Foundation survey. LTC insurance ownership 10–15% of Americans 65+; ~70% of those over 65 will need long-term care.

- **[LIMRA 2026]** — LIMRA / EY, hybrid life-with-LTC product research, 2026.
- **[Lancet Commission 2024]** — Livingston G, et al., *Lancet* Standing Commission on dementia prevention, intervention, and care, 2024 update. Fourteen modifiable risk factors; up to ~45% of dementia cases theoretically preventable or delayable. **This is a population-attributable fraction — a population-wide ceiling under conditions that will never obtain — not an individual’s expected risk reduction. Never present 45% as a personal number. 30% is this book’s conservative default for any individual relative-risk-reduction claim.** alzheimer-europe.org
- **[US POINTER, JAMA 2025]** — Baker LD, et al. US POINTER. *JAMA* 2025;334(8):681–691. doi:10.1001/jama.2025.12923. 2,111 participants, mean age 68.2, 2-year RCT; both structured and self-guided multidomain lifestyle interventions improved global cognition, structured significantly more, consistent across age, sex, ethnicity, cardiovascular status, and APOE-ε4. *Improvement/protection-from-decline finding — not a prevention finding.* jamanetwork.com/journals/jama/fullarticle/2837046
- **[Ngandu et al., Lancet 2015]** — Ngandu T, et al. A 2 year multidomain intervention (FINGER). *Lancet*

- 2015;385(9984):2255–2263. doi:10.1016/S0140-6736(15)60461-5. 2-year multidomain lifestyle intervention improved/maintained global cognition vs. control; the design basis for US POINTER.
- **[Iso-Markku et al., BJSM 2022]** — Iso-Markku P, et al. Physical activity as a protective factor for dementia and Alzheimer’s disease: systematic review, meta-analysis and quality assessment of cohort and case-control studies. *Br J Sports Med* 2022;56(12):701–709. doi:10.1136/bjsports-2021-104981. All-cause RR 0.80 (0.77–0.84); AD 0.86 (0.80–0.93); vascular 0.79 (0.66–0.95).
 - **[Feter et al., Lancet Public Health 2026]** — Feter N, Iso-Markku P, et al. Domain-specific physical activity and dementia risk: systematic review and meta-analysis. *Lancet Public Health* 2026;11(9):e650–e664. doi:10.1016/S2468-2667(26)00142-8. Leisure-time RR $\approx$ 0.76; occupational RR $\approx$ 1.20.
 - **[physical activity meta-analyses — SUPERSEDED tag, retained for reference]** — Pooled relative risk for all-cause dementia with higher physical activity $\approx$ 0.80 (95% CI 0.77–0.84); Alzheimer’s-specific $\approx$ 0.86; vascular dementia $\approx$ 0.79; leisure-time activity specifically $\approx$ 0.76. Occupational physical activity showed a slightly *increased* risk (RR $\approx$ 1.20) in one large analysis — the benefit is domain-specific. *Observational pooled estimates; individual studies to be cited in full at manuscript lock.*
 - **[Qi et al., PLOS ONE 2024]** — Qi X, et al. *PLOS ONE* 2024;19(11):e0310964. doi:10.1371/journal.pone.0310964.

Hazard ratios for subsequent dementia by age at type 2 diabetes diagnosis (reference: diagnosis at ≥ 70): 60–69 $\rightarrow$ HR 1.70; 50–59 $\rightarrow$ HR 1.72; before 50 $\rightarrow$ HR 1.90. Obesity plus diabetes diagnosed before 50 $\rightarrow$ HR 3.05 vs. non-obese diagnosed ≥ 50 . Midlife metabolically unhealthy obesity in women $\rightarrow$ HR 1.62.

ncbi.nlm.nih.gov/pmc/articles/PMC11559992

- **[premium basket pricing, 2025–2026]** — Composite of published 2025–2026 US consumer pricing: mid-tier gym membership \$30–\$120/mo; personal trainer once weekly \$160–\$560/mo; organic/grass-fed grocery premium for a two-person household \$120–\$250/mo; quality supplement stack \$100–\$200/mo; single compounded prescription therapy category \$150–\$350/mo. *Ranges, not quotes. Re-verify at manuscript lock; regional variation is large.*
- **[Xie et al., Science 2013]** — Xie L, Kang H, Xu Q, et al., “Sleep Drives Metabolite Clearance from the Adult Brain,” *Science*, 18 Oct 2013. Glymphatic clearance of interstitial solutes, including amyloid- β , is markedly increased during sleep relative to waking. *Rodent study; the human extension is inferential — state it as mechanism, not as a human outcome finding.*
- **[Dominy et al., Sci. Adv. 2019]** — Dominy SS, Lynch C, Ermini F, et al., “*Porphyromonas gingivalis* in Alzheimer’s disease brains: Evidence for disease causation and treatment with small-molecule inhibitors,” *Science Advances*, 23 Jan 2019. Periodontal pathogen and its gingipain pro-

teases identified in Alzheimer's brain tissue. *Association plus a proposed mechanism; causation is argued by the authors, not established. The study was funded by Cortexyme, a company co-founded by two of the paper's senior authors and developing a ginglypain inhibitor — note that funder relationship explicitly, not just “industry funding.”*

- **[Leong et al., *Lancet* 2015]** — Leong DP, Teo KK, Rangarajan S, et al., “Prognostic value of grip strength: findings from the Prospective Urban Rural Epidemiology (PURE) study,” *The Lancet*, 2015. ~140,000 participants, 17 countries; grip strength was a stronger predictor of all-cause and cardiovascular mortality than systolic blood pressure.
- **[GBD Alcohol Collaborators, *Lancet* 2018]** — GBD 2016 Alcohol Collaborators, “Alcohol use and burden for 195 countries and territories, 1990–2016,” *The Lancet*, 22 Sep 2018. Level of consumption minimizing total health loss = zero. *A population-level burden analysis, not an individual risk prediction.*
- **[Boyle et al., *Arch. Gen. Psychiatry* 2010]** — Boyle PA, Buchman AS, Barnes LL, Bennett DA, “Effect of a purpose in life on risk of incident Alzheimer disease and mild cognitive impairment in community-dwelling older persons,” *Archives of General Psychiatry*, 2010 (Rush Memory and Aging Project). High purpose-in-life score associated with ~2.4× lower risk of incident Alzheimer's vs. low score, adjusted for age, sex, education, depressive

symptoms, and baseline cognition. Later work from the same cohort reports reduced clinical expression of equivalent neuropathology. *Observational; purpose is self-reported on a validated scale and confounding with baseline function cannot be fully excluded.*

- **[EPA 1989; Klepeis et al., JEAEE 2001]** — U.S. EPA, *Report to Congress on Indoor Air Quality, Vol. II*, EPA/400/1-89/001C, 1989 (indoor pollutant levels 2–5× outdoor, TEAM studies). Klepeis NE, Nelson WC, Ott WR, et al., “The National Human Activity Pattern Survey (NHAPS),” *J Expo Anal Environ Epidemiol* 2001;11(3):231–252 (Americans spend ≈87% of time indoors, EPA-funded; commonly rounded to “~90%,” including on EPA’s own current indoor-air pages).
- **[Ebrahim et al., Alcohol Clin. Exp. Res. 2013]** — Ebrahim IO, Shapiro CM, Williams AJ, Fenwick PB, “Alcohol and Sleep I: Effects on Normal Sleep,” *Alcoholism: Clinical and Experimental Research*, 2013;37(4):539–549. doi:10.1111/acer.12006. Meta-analysis of 27 studies: increased first-half slow-wave sleep; REM suppression in the first half is dose-dependent (significant at high doses); total-night REM percentage reduced at moderate-to-high doses; second-half sleep disruption increases as blood alcohol clears. No single percentage-reduction figure is supported by this literature — the companion book’s “20–50% REM cut” is not carried into this book.
- **[Gershon & Tack, Gastroenterology 2007; Eisenhofer et al., JCEM 1997; Berthoud &**

- Neuhuber, Auton. Neurosci. 2000]** — Gershon MD, Tack J, *Gastroenterology* 2007;132(1):397–414 (enterochromaffin cells $\approx$ 95% of body serotonin); Eisenhofer G, et al., *J Clin Endocrinol Metab* 1997;82(11):3864–3871 (mesenteric/GI production accounts for roughly half of total-body dopamine); Berthoud HR, Neuhuber WL, *Auton Neurosci* 2000;85(1–3):1–17 (vagus $\approx$ 80% afferent, 20% efferent fibers). The body text’s vagal figure is stated as “roughly 80%,” consistent with the anatomical literature rather than the commonly repeated “90%.”
- **[Wrann et al., Cell Metab. 2013; Moon et al., Cell Metab. 2016]** — Wrann CD, White JP, Salogiannis J, et al., *Cell Metabolism* 2013;18(5):649–659, doi:10.1016/j.cmet.2013.09.008 (exercise $\rightarrow$ PGC-1 α /FND-C5/irisin $\rightarrow$ hippocampal BDNF; mouse). Moon HY, Becke A, Berron D, et al., *Cell Metabolism* 2016;24(2):332–340, doi:10.1016/j.cmet.2016.05.025 (cathepsin B required for exercise-induced neurogenesis in mice; plasma CTSB rise correlates with memory function in monkeys and humans — the human leg of this evidence). Rodent mechanism, human evidence limited to CTSB correlation; stated in the body text as “animal models and early human work,” not settled human causation.
 - **[Wilkinson et al., Ageing Res. Rev. 2018]** — Wilkinson DJ, Piasecki M, Atherton PJ, “The age-related loss of skeletal muscle mass and function,” *Ageing Research Reviews*, 2018;47:123–132. doi:10.1016/j.arr.2018.07.005. Muscle mass loss $\approx$ 1%/year

beginning in midlife, accelerating with age; cumulative loss $\approx 50\%$ by the 8th–9th decade in severe cases. Body text restated as “roughly one percent, accelerating toward two percent in later decades” rather than a flat “1–2% after 50.”

- **[Montorsi et al., Eur. Urol. 2003; Thompson et al., JAMA 2005]** — Montorsi F, et al., *European Urology* 2003;44(3):360–365, doi:10.1016/S0302-2838(03)00305-1 (ED preceded CAD symptoms in most cases; mean interval ≈ 39 months, range 1–165 months; artery-size hypothesis: penile $\approx 1\text{--}2$ mm vs. coronary $\approx 3\text{--}4$ mm). Thompson IM, et al., *JAMA* 2005;294(23):2996–3002, doi:10.1001/jama.294.23.2996 (incident ED, HR 1.25 for subsequent cardiovascular events in a 9,457-man cohort). The book’s lead time is restated as “commonly two to three years, and sometimes longer,” consistent with the primary cohort’s mean lead time (≈ 39 months, wide range).
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